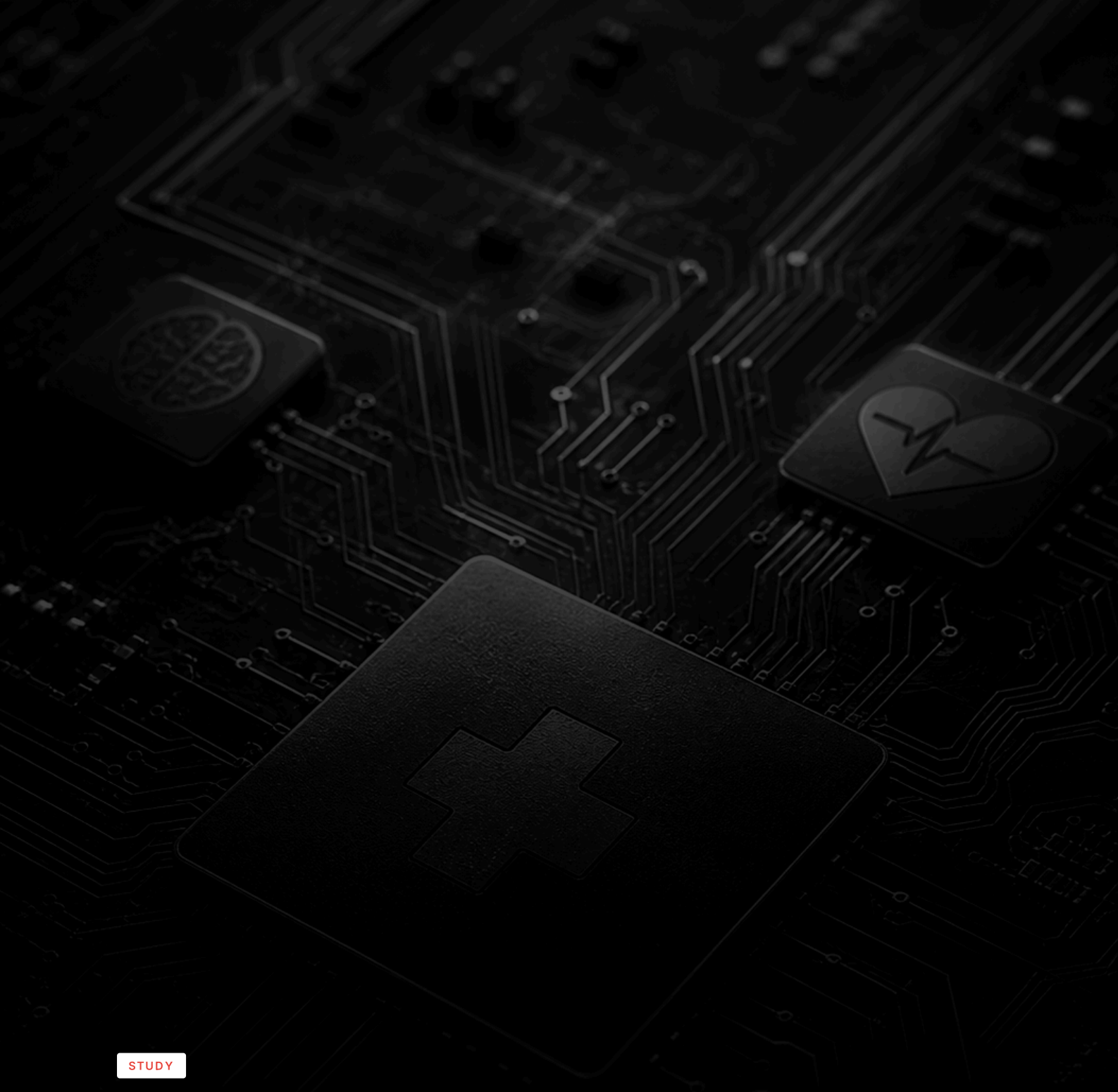




STUDY

CompuGroup Medical: Competitive Benchmarking

This comprehensive report benchmarks CompuGroup Medical against top competitors like Epic Systems, Oracle Health, and European rivals. It offers strategic insights and financial analyses pertinent to investors and M&A professionals.

STUDY REPORT

CompuGroup Medical: Competitive Benchmarking

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• Executive Summary

Finding 1 — High-quality revenue base with FY2025 growth outlook

CompuGroup Medical (CGM) reported FY2024 revenue of EUR 1.154 billion, of which 74% was recurring, underscoring strong cash-flow visibility. The company projects FY2025 revenue of EUR 1.213 billion, reinforcing its appeal to investors seeking predictable recurring revenue.

Finding 2 — Large, fast-growing market supports expansion thesis

The global health IT market is expected to grow at a mid-teens rate through 2026, with projected market size ranging from USD 403 billion to USD 999 billion. Key demand drivers are AI workflow automation, cloud migration, and modernization of pharmacy and lab systems, with clinician-productivity software attracting major investment.

Finding 3 — European leadership tempered by global scale disadvantage

CGM is positioned as a leading health IT vendor in Europe, with strategic strength in the DACH region and Western Europe and a diversified portfolio across healthcare settings. However, the company operates at a smaller scale than major U.S. competitors such as Epic, Oracle Health, and athenahealth, particularly in acute care and ambulatory markets.

Finding 4 — Recurring model is a competitive advantage, but execution gaps remain

Relative to peers, CGM benefits from stronger recurring revenue visibility and a strategic focus on hospital information and pharmacy systems modernization. At the same time, the report highlights pressure to accelerate cloud services, improve innovation capabilities, strengthen customer advocacy, and lift profitability.

Finding 5 — Cloud, AI, and localized installed-base monetization are primary value levers

The report identifies cloud migration and AI integration as core strategic priorities, with growth opportunities tied to monetizing CGM's installed base through workflow enhancements and localization. These initiatives are seen as central to improving market visibility, valuation, and competitive positioning.

Finding 6 — M&A attractiveness depends on discipline and platform convergence

The market is shifting from scale-driven consolidation toward platform densification built on recurring workflows and AI adoption, positioning CGM as an attractive asset given its recurring revenue base. However, the report stresses that better product integration, clearer M&A criteria, improved platform convergence, and tighter capital allocation discipline are required to unlock valuation upside.

1

Introduction and Scope

1

Introduction and Scope

This report frames CompuGroup Medical as a scaled European health IT platform with meaningful recurring revenue, a broad installed base across ambulatory, hospital, pharmacy, and adjacent workflow software, and a strategic position in a consolidating sector shaped by interoperability mandates, cloud migration, AI enablement, and provider cost pressure. CGM generated EUR 1.154 billion of revenue in FY2024, with recurring revenue of EUR 853.9 million or 74 percent of sales and an adjusted EBITDA margin of 19 percent, while company disclosures for FY2025 indicate a return to growth with revenue of EUR 1.213 billion and adjusted EBITDA of EUR 240 million, reinforcing its relevance for public market investors and strategic buyers assessing durable cash flow, installed-base monetization, and portfolio optionality.^{17 14 20}

The scope of the broader benchmarking study is global, but weighted toward the two arenas that matter most for valuation and strategic positioning: DACH and Western Europe, where CGM has structural density and local workflow relevance, and the United States, where scale economics, KLAS visibility, and acute care concentration are set by Epic, Oracle Health, athenahealth, MEDITECH, eClinicalWorks, and other major vendors. The analytical framework combines company filings, investor materials, KLAS market intelligence, regulatory context, and competitor disclosures to compare CGM against both global suite vendors and regional European specialists such as Dedalus and NEXUS, with emphasis on revenue quality, market share proxies, customer satisfaction, and M&A optionality.^{26 40 36}

1.1

Overview of CompuGroup Medical

CompuGroup Medical is a Germany-based health IT vendor focused on ambulatory information systems, hospital information systems, pharmacy information systems, and connected workflow services, making it one of the few listed European platforms with material exposure across multiple care settings. For investors, the core attraction is not only sector exposure to digitization in healthcare, but also the company's high recurring revenue mix, installed-base stickiness, and capacity to cross-sell software, connectivity, revenue cycle, and data-enabled services across fragmented provider markets.^{17 13}

- Scale and revenue base: CGM reported FY2024 revenue of EUR 1,153.987 million, recurring revenue of EUR 853.904 million, and an adjusted EBITDA margin of 19 percent. Company disclosures for FY2025 indicate revenue of EUR 1.213 billion, organic growth of 5 percent, and adjusted EBITDA of EUR 240 million, signaling renewed operating momentum after a softer 2024.^{17 14 20}
- Revenue quality: In FY2024, recurring revenue represented 74 percent of group sales, up from 69 percent in FY2023, which is a critical valuation support factor because it improves visibility, resilience, and financing capacity in a sector where implementation cycles can be uneven.¹⁷
- Segment relevance: CGM's portfolio spans ambulatory, hospital, and pharmacy software, allowing benchmarking against both broad suite vendors and niche specialists rather than a single peer set. That multi-segment exposure is strategically important because investor outcomes depend on whether CGM can defend local market positions while modernizing products toward cloud, AI, and platform interoperability.^{17 13}

- Investment relevance: CGM sits at the intersection of defensive healthcare software characteristics and strategic transformation risk. That combination makes it a credible public equity investment case and a relevant M&A asset for buyers seeking European provider IT scale, recurring maintenance streams, and country-specific workflow penetration.^{20 17}

1.2

Scope of Benchmarking

The benchmarking perimeter is intentionally broader than the conventional EHR peer set because CGM competes across ambulatory, hospital, pharmacy, laboratory-adjacent, connectivity, and revenue cycle workflows. The report therefore compares CGM with global leaders such as Epic, Oracle Health, athenahealth, eClinicalWorks, and MEDITECH, while also incorporating European rivals including Dedalus, NEXUS, and other regionally relevant vendors where they influence procurement, pricing, or consolidation dynamics.^{26 40 36}

- Geographic scope: Global by design, with primary analytical depth in DACH and Western Europe, where CGM's heritage, installed base, and regulatory familiarity are strongest, and in the United States, where competitive standards are set by large-scale acute and ambulatory platforms with stronger KLAS visibility and larger capital budgets.^{17 40}
- Segment scope: The study covers ambulatory EHR and practice management, acute care EHR and patient accounting, hospital information systems, pharmacy systems, interoperability and connectivity, and selected revenue cycle and outsourced services where CGM or peers monetize adjacent workflows.^{17 13}
- Competitive scope: Epic, Oracle Health, athenahealth, eClinicalWorks, and MEDITECH define the U.S. comparison set for scale, customer satisfaction, and acute or ambulatory share. Dedalus and NEXUS are essential European comparators because they shape hospital procurement and regional consolidation economics, while additional local vendors are included where country-level relevance is material.^{26 27 36}
- Benchmarking lens: The report evaluates not just absolute scale, but also revenue quality, margin structure, R&D intensity, installed-base durability, customer sentiment, and strategic optionality. This is necessary because CGM's investment case is more comparable to a recurring software platform with regional defensibility than to a pure-play U.S. mega-vendor.^{17 37}

1.3

Methodology, Data Sources, and Analytical Framework

The report uses a triangulated benchmarking methodology designed for investor and M&A use rather than product marketing comparison. Primary emphasis is placed on audited filings, investor disclosures, and recognized market intelligence, with secondary sources used to fill gaps in private-company visibility, customer sentiment, and regional market structure.^{17 26 37}

- Primary sources: CGM annual reports and investor communications, public filings from listed peers such as Oracle and NEXUS, and official company materials from private vendors where available. These sources anchor revenue, margin, recurring revenue, segment mix, and strategic disclosures.^{17 20 36 37}
- Secondary sources: KLAS is used for customer satisfaction, category leadership, and acute care market-share directionality, including confirmation that 2025 net-new acute care EHR decisions validated by KLAS were concentrated among a small set of vendors. Additional trade

and industry sources are used selectively where they summarize KLAS outputs or company announcements not otherwise accessible in full.^{26 40 27}

- Analytical approach: The framework combines horizontal peer comparison and vertical segment analysis. Horizontal comparison assesses scale, profitability, recurring revenue, and valuation relevance across vendors, while vertical analysis tests CGM's position within ambulatory, hospital, pharmacy, and adjacent workflow niches by geography and customer type.^{17 36}
- Treatment of data gaps: Private vendors such as Epic, athenahealth, eClinicalWorks, and Dedalus do not provide the same level of public financial granularity as listed peers, so the report distinguishes between disclosed figures, third-party estimates, and directional market evidence. Where direct comparability is limited, the analysis prioritizes installation share, KLAS standing, product breadth, and strategic control points rather than forcing false precision.^{26 25 32}
- Research note on source selection: Initial arXiv screening did not yield material directly useful for company, market-share, or investment benchmarking of health IT vendors through mid-2026. The relevant evidence for this section therefore comes from company filings, investor materials, and healthcare IT market intelligence rather than academic preprints.^{2 4 3}

2

Global Health IT Market Landscape

2

Global Health IT Market Landscape

The global health IT market entered 2026 with strong structural growth but wide dispersion in reported market size because research firms use different scope definitions, ranging from core provider and payer IT to broader digital health and services. Even with that caveat, the directional conclusion is consistent: the market is expanding at a mid-teens rate, with 2026 global estimates spanning roughly USD 403 billion to USD 999 billion and long-range growth expectations of about 16 percent CAGR, supported by provider digitization, AI-enabled workflow automation, cloud migration, interoperability mandates, and pharmacy and lab modernization.^{49 50 53}

For investors and M&A professionals, the more important takeaway than any single top-down number is where value is accruing. Growth is concentrating in software and data layers that improve clinician productivity, automate revenue and administrative workflows, enable cross-setting data exchange, and create AI-ready longitudinal records, while legacy on-premises estates in hospital, ambulatory, pharmacy, and laboratory environments remain the monetizable installed base that incumbents such as CGM, Dedalus, Oracle Health, Epic, NEXUS, and regional specialists are trying to migrate rather than replace outright.^{58 59 72}

2.1

Market Size, Growth Trajectory, and Key Trends

Reported market size varies materially by taxonomy, but the growth signal is unambiguous. Fortune Business Insights values the global healthcare IT market at USD 354.0 billion in 2025 and USD 402.7 billion in 2026, implying a 2026 to 2034 CAGR of 16.65 percent, while Grand View Research places the market at USD 866.4 billion in 2025 and USD 998.8 billion in 2026 with a 16.2 percent CAGR through 2033; 360iResearch estimates USD 531.5 billion for 2026 after USD 464.4 billion in 2025.^{49 50 53}

- Interpretation for investors: The spread in absolute market size reflects scope differences, not disagreement on direction. Across methodologies, health IT remains one of the faster-growing healthcare software categories into 2026, which supports premium valuation frameworks for vendors with recurring revenue, migration pathways, and AI monetization optionality.^{49 50}
- Demand drivers: Growth is being pulled by provider labor shortages, documentation burden, remote and ambulatory care expansion, payer and provider interoperability requirements, and the need to modernize fragmented core systems rather than continue funding bespoke interfaces and manual workflows.^{58 59 78}
- AI trend: AI is shifting from point pilots to embedded workflow tooling in documentation, summarization, coding, inbox management, and decision support. An August 2026 arXiv paper notes that an estimated 70 to 80 percent of healthcare AI pilots fail to scale because of governance, data fragmentation, and integration gaps, which is strategically favorable for incumbent platform vendors that control workflow, identity, and data access layers.⁴¹
- Cloud trend: Cloud adoption is no longer a discretionary architecture choice. It is increasingly the prerequisite for continuous deployment, API-based interoperability, AI model orchestration, and lower total cost of ownership, especially in ambulatory and mid-market hospital settings.⁵⁰

58

- Capital markets implication: The market is rewarding vendors that can convert maintenance-heavy installed bases into subscription, platform, and transaction revenue. In practical terms, this favors companies with strong migration economics and penalizes vendors whose legacy estates require high service intensity without clear cloud or AI upsell paths.^{59 58}

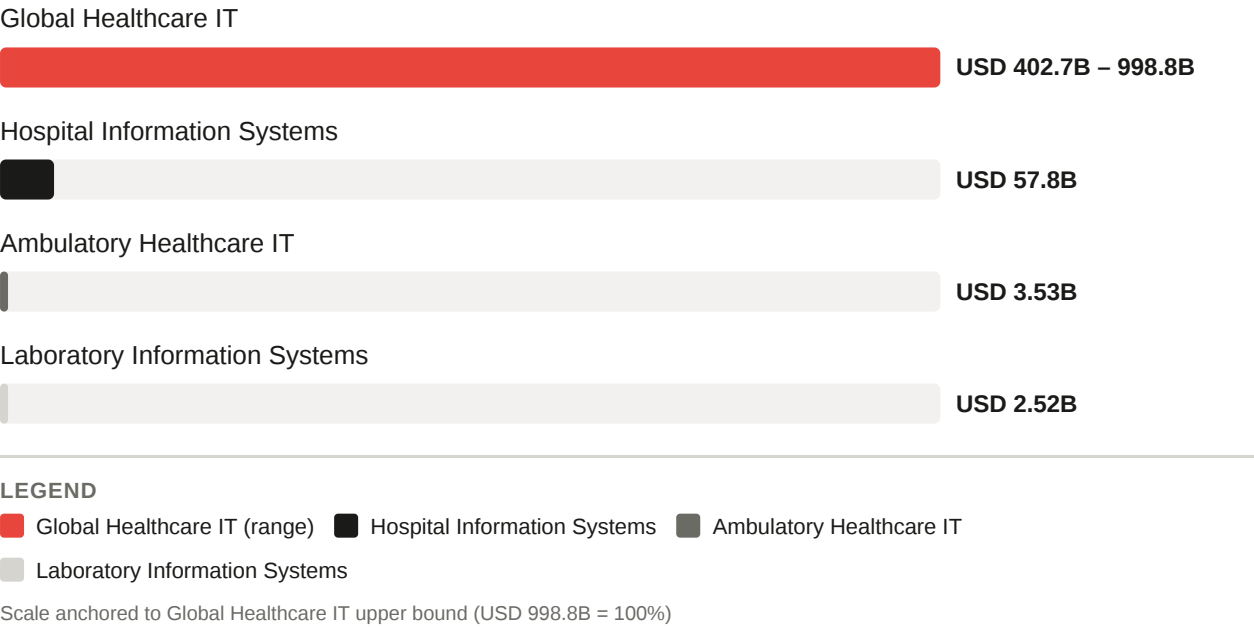
2.2

Segmentation: Ambulatory, Hospital, Pharmacy, and Lab IT

Title — Selected 2026 health IT segment indicators

SEGMENT	2026 MARKET INDICATOR	STRATEGIC READ-THROUGH
Global healthcare IT	USD 402.7B to USD 998.8B	Broad market growing at roughly mid-teens CAGR, depending on scope.
Hospital information systems	USD 57.8B	Large installed base, slower growth than broader health IT, but high switching costs.
Ambulatory healthcare IT	USD 3.53B	Smaller defined niche in this taxonomy, supported by physician office digitization and workflow automation.
Laboratory information systems	USD 2.52B	Steady mid-single-digit growth, mission-critical but more mature than AI and cloud layers.

Source: 49 52 51 65



- Ambulatory IT: Ambulatory systems remain a structurally attractive segment because they combine high logo counts, recurring subscription potential, and growing demand for integrated EHR, practice management, patient engagement, telehealth, and AI documentation tools. Research and Markets estimates the ambulatory healthcare IT market at USD 3.53 billion in 2026, up from USD 3.32 billion in 2025, or 6.2 percent growth, though this appears to capture a narrower product definition than broader ambulatory software ecosystems.⁵¹

- Hospital IT: Hospital information systems remain the largest of the four named care setting segments in absolute software value and the most defensible from a switching cost perspective. Grand View Research values the global hospital information system market at USD 57.8 billion in 2026, up from USD 54.7 billion in 2025, with a 6.4 percent CAGR through 2033, indicating a mature but still expanding core market where replacement cycles are selective and modernization spend increasingly targets cloud, interoperability, and AI overlays.⁵²
- Pharmacy IT: Pharmacy IT is benefiting from ePrescription growth, medication workflow digitization, retail and outpatient fulfillment integration, and tighter reimbursement and prior authorization workflows. While a clean global 2026 pharmacy IT total was not located in a reliable primary or quasi-primary source during this search, pharmacy is explicitly included as an end-user growth vector in major healthcare IT market models, and its strategic importance is rising because medication workflows are becoming a key interoperability and automation battleground.^{49 57}
- Lab IT: Laboratory information systems are smaller in absolute market size but remain operationally critical and sticky. BCC Research shows the LIS segment reaching USD 2.52 billion by 2026 from USD 1.81 billion in 2021, implying a 6.9 percent CAGR, which suggests dependable but comparatively moderate expansion relative to broader health IT and AI-enabled workflow categories.⁶⁵
- Segment contribution to growth: Hospitals still anchor absolute spending, but ambulatory, pharmacy, and lab environments are increasingly where modular innovation is commercialized first because deployment cycles are shorter and ROI is easier to evidence. For CGM and European peers, this matters because cross-setting portfolios can capture both the durable economics of hospital core systems and the faster innovation cadence of ambulatory and pharmacy workflows.^{59 58}

2.3

Macro Forces Shaping Competitive Dynamics

The competitive landscape in 2026 is being shaped less by feature parity in core records and more by who can operationalize compliant data liquidity. AI, cloud, interoperability, and regulation are now interdependent rather than separate themes, and vendors that cannot align all four are at risk of margin compression, slower bookings, or installed base attrition.^{58 59 41}

- AI as a platform test: Generative AI is moving procurement criteria toward workflow orchestration, data quality, and governance. The practical implication is that incumbents with embedded clinical workflows and broad data access can defend share, but only if they can productize AI safely and at scale rather than rely on isolated copilots.^{41 44}
- Cloud economics: Cloud migration is reshaping competitive dynamics by lowering deployment friction, enabling continuous upgrades, and supporting API-first architectures. It also changes revenue recognition and valuation quality by increasing subscription and managed service mix, which is strategically relevant for investors comparing legacy license vendors with cloud-native or cloud-transitioning peers.^{50 58}
- Interoperability pressure in the United States: U.S. regulation continues to push vendors toward standardized data exchange and lower information friction. ONC's HTI rule set updates certification and information sharing requirements, while CMS states that under the Interoperability and Prior Authorization Final Rule, impacted payers must provide specific reasons for denied prior authorization decisions beginning in 2026, increasing the value of integrated payer-provider workflow software.^{74 78}

- Interoperability pressure in Europe: The European Health Data Space Regulation was published on March 5, 2025 and entered into force on March 26, 2025, with phased implementation beginning later in the decade. Even before full application, EHDS is already influencing procurement by favoring vendors that can support standardized exchange, patient access, and secondary use governance across member states.^{69 70}
- AI regulation in Europe: The European Commission states that the EU AI Act entered into force on August 1, 2024, with phased applicability thereafter. For health IT vendors, this raises compliance costs but also creates a barrier to entry that may advantage scaled incumbents able to fund documentation, risk management, and post-market controls across clinical AI use cases.^{71 82}
- Macro and budget backdrop: OECD analysis indicates health systems remain under pressure to improve efficiency, workforce productivity, and data governance while managing constrained budgets and reform agendas. That environment supports ROI-led purchasing, bolt-on automation, and consolidation around fewer strategic platforms, which is favorable for vendors with recurring revenue and cross-setting breadth but challenging for subscale point solution providers.^{62 58}

3

CompuGroup Medical: Company Profile and Strategic Positioning

3

CompuGroup Medical: Company Profile and Strategic Positioning

CompuGroup Medical enters mid 2026 as a scaled European health IT platform with a multi-setting portfolio spanning ambulatory, hospital, pharmacy, laboratory, connectivity, and adjacent workflow services. For investors, the defining features are a high recurring revenue base, broad regulatory and workflow entrenchment in DACH and Western Europe, and a product modernization agenda centered on cloud delivery, AI-enabled automation, and platform interoperability, even as the company continues to manage leverage, portfolio complexity, and execution risk in hospital modernization programs.^{100 17}

3.1

Business Model, Revenue Structure, and Segment Breakdown

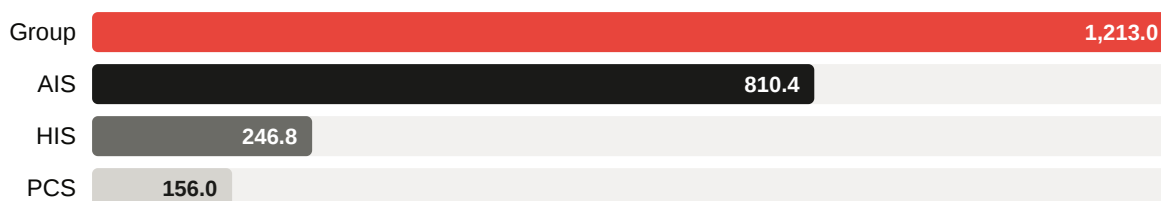
CGM's model is built around mission-critical software sold into fragmented provider and pharmacy markets, with monetization split between recurring maintenance, leases, SaaS, transaction-like services, and more cyclical one-time license, hardware, and project revenues. The strategic implication is that AIS and PCS behave more like installed base software annuities, while HIS carries lower margin but higher strategic value because it anchors hospital workflows and creates cross-sell opportunities into revenue cycle, interoperability, and clinical documentation.^{102 100}

Title — CGM segment mix and profitability

SEGMENT	2025 REVENUE EUR M	RECURRING SHARE	ADJUSTED EBITDA MARGIN
AIS	810.4	66%	23%
HIS	246.8	70%	13%
PCS	156.0	65%	32%
Group	1,213.0	73%	16%

Source: ¹⁰⁰

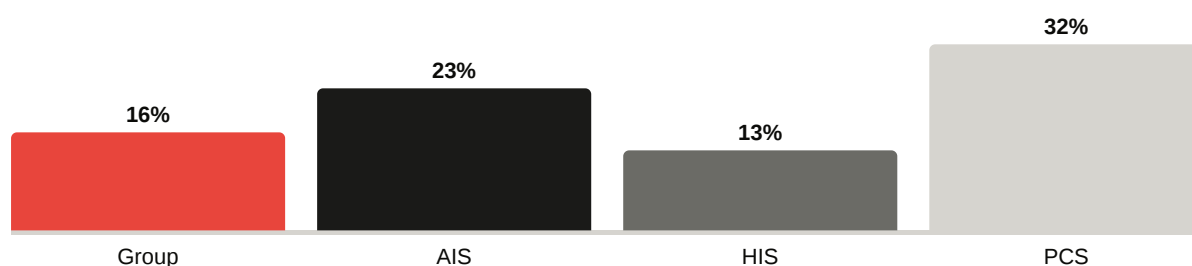
2025 REVENUE (EUR M)



RECURRING SHARE (%)



ADJUSTED EBITDA MARGIN (%)



- AIS: Ambulatory Information Systems generated EUR 810.4 million of 2025 revenue, or roughly two thirds of group sales by inference, with recurring revenue of EUR 531.7 million and a 66 percent recurring mix. This remains the economic core of CGM because it combines large physician office exposure, maintenance and SaaS-like cash flows, and the clearest path to AI and cloud upsell through CGM ONE.¹⁰⁰
- HIS: Hospital Information Systems delivered EUR 246.8 million of 2025 revenue, 10 percent organic growth, recurring revenue of EUR 173.0 million, and a 70 percent recurring share. The segment remains more project-oriented and tender-driven than AIS or PCS, but it is strategically important because CGM is using it to position G3-based products, SAP IS-H successor functionality, and hospital workflow automation in markets where replacement cycles are opening.^{100 102}
- PCS: Pharmacy Information Systems produced EUR 156.0 million of 2025 revenue, 7 percent organic growth, recurring revenue of EUR 102.0 million, and a 32 percent adjusted EBITDA margin. PCS is smaller in absolute scale but disproportionately valuable because pharmacy software and related data services carry strong retention characteristics and attractive unit economics.¹⁰⁰
- Revenue quality trend: Group recurring revenue rose from EUR 853.9 million in 2024 to EUR 880.5 million in 2025, while the recurring share moved from 74 percent in 2024 to about 73 percent in 2025 because faster growth in project and other non-recurring activity diluted the mix. For valuation, this is still a high-quality revenue profile relative to many legacy European software assets, but it also shows that hospital and transformation work can temporarily reduce the apparent subscription intensity.^{102 100}

- **Margin structure:** Segment profitability remains uneven, with AIS at 23 percent adjusted EBITDA margin, HIS at 13 percent, and PCS at 32 percent in 2025. This spread matters strategically because value creation depends on whether CGM can lift HIS economics through product standardization and cloud delivery without sacrificing the installed base defensibility that supports the broader platform.¹⁰⁰
- **Financial structure:** Net debt stood at EUR 772.8 million at year-end 2024 and leverage was 3.48x on CGM's LTM definition, up from 2.75x in 2023. That balance sheet profile does not undermine the recurring cash flow thesis, but it raises the premium on disciplined capital allocation, successful product migration, and margin recovery in 2026 and beyond.¹⁰²

3.2

Geographic Footprint, Installed Base, and Recurring Revenue Profile

CGM's geographic profile is unusually broad for a European listed health IT vendor, but economically concentrated in Germany and Western Europe. The company had offices in 19 countries and distributed products in 60 countries as of the 2024 and 2025 reporting periods, giving it a diversified route to market while preserving a home market bias that supports regulatory fit, local workflow depth, and customer retention.^{102 100}

- **Regional revenue concentration:** In 2025, Germany contributed EUR 684.5 million of third-party revenue, or about 56 percent of group sales, while Western Europe contributed EUR 356.1 million, North America EUR 133.4 million, Eastern Europe EUR 33.5 million, and the rest of the world EUR 5.5 million. This mix confirms that CGM is fundamentally a DACH and Western European platform with a smaller but still relevant U.S. business, rather than a balanced global vendor.¹⁰⁰
- **Installed base logic:** CGM's customer backbone spans physicians, dentists, hospitals, pharmacies, laboratories, insurers, and pharmaceutical companies, which creates a cross-setting installed base that is broader than a pure EHR vendor and more locally embedded than most U.S. acute care peers. For investors, that breadth matters because it supports cross-sell into connectivity, patient communication, document automation, and data services without requiring greenfield customer acquisition in every workflow.¹⁰²
- **Recurring revenue resilience:** Group recurring revenue was EUR 853.9 million in 2024 and EUR 880.5 million in 2025, representing 74 percent and about 73 percent of revenue respectively. The durability of this base is reinforced by maintenance contracts, software leases, SaaS, and recurring service models that are embedded in daily provider and pharmacy operations.^{102 100}
- **Segment-level recurrence:** In 2025, recurring revenue shares were 66 percent in AIS, 70 percent in HIS, and 65 percent in PCS. The notable point is that even the hospital segment, despite its project orientation and public tender exposure, now carries a majority recurring profile, which improves visibility relative to traditional perpetual license hospital software models.¹⁰⁰
- **Market-specific stickiness:** Germany remains the strongest retention anchor because CGM is deeply exposed to national infrastructure and workflow layers such as e-prescription processing, telematics, and ambulatory practice software. The company noted that more than 90,000 medical facilities in Germany issue e-prescriptions and over 17,000 pharmacies process them, a market structure that favors incumbents with embedded connectivity and compliance tooling.¹⁰²

- Portfolio implication: Compared with U.S. mega vendors that dominate acute care but have narrower exposure to pharmacy and country-specific ambulatory workflows in Europe, CGM's installed base is less concentrated by customer type and more diversified by care setting. That diversification supports downside resilience, but it also increases product and go-to-market complexity, which can weigh on execution and valuation if modernization is uneven.^{102 100}

3.3

Strategic Priorities and Innovation Pipeline

CGM's current strategy is less about expanding into entirely new categories and more about upgrading the monetization and defensibility of its installed base. Through 2025 and into 2026, management has emphasized cloud migration, AI-embedded workflow tools, open platform architecture, and operational excellence, with the stated 2026 outlook calling for low- to mid-single-digit organic revenue growth and a slight increase in cash EBITDA margin.^{92 100}

- AI as product layer: CGM states it has been strategically investing in artificial intelligence, machine learning, and large language models since 2023, with the effort intensified in 2025. Existing examples include the CGM ONE telephone assistant, DokuAssistant, CGM HIS Assistant, and the U.S. ambient AI product CGM AMBI, indicating that AI is being deployed first in documentation, communication, and administrative workflow compression rather than high-risk clinical decisioning.^{100 102 93}
- Cloud migration in AIS: The flagship ambulatory modernization program is CGM Praxis on the cloud-based CGM ONE platform, which management describes as a new cloud AIS and end-to-end solution for medical practices. Add-ons such as TelefonAssistent, Self CheckIn, AmbulApps, and DokuAssistent show a deliberate move from classic practice software toward modular automation services that can raise ARPU and improve retention.^{100 92}
- Hospital platform strategy: In HIS, CGM is expanding the CGM CLINICAL Suite on open CGM G3 technology and positioning CGM CLINICAL RCM as a successor path around the SAP IS-H transition. This is strategically important because it gives CGM a time-sensitive opening to capture modernization budgets in German-speaking hospital markets where legacy administrative and billing architectures are being reconsidered.^{92 102}
- Pharmacy digitization: In PCS, CGM is pushing cloud-based pharmacy software through CGM STELLA, first highlighted in Italy and reiterated in 2026 product communications. The investment case here is that pharmacy remains a high-retention vertical where cloud delivery can reduce support burden, improve update cadence, and create a platform for adjacent data and workflow services.^{102 92}
- Security and compliance as enablers: CGM highlighted C5 attestation for its cloud services in Germany in the 2024 annual report, framing security assurance as a prerequisite for healthcare cloud adoption. In European provider IT, this is not a marketing detail but a commercial gating factor, especially for AI-enabled and hosted workflows in regulated ambulatory and hospital settings.¹⁰²
- Innovation spend and execution burden: CGM employed an average of 2,759 R&D staff in 2025 and reported EUR 274 million of R&D expense, up from EUR 255 million in 2024. That level of spend is strategically necessary to modernize a broad portfolio, but for investors it also means the key question is not whether CGM is investing enough, but whether it can convert that spend into faster cloud migration, better segment margins, and a clearer platform narrative versus more focused competitors.¹⁰⁰

- Strategic read-through: CGM's innovation pipeline is credible because it is tied to existing workflows and installed customers rather than speculative moonshots. The competitive edge will depend on speed of rollout and commercial packaging, since rivals such as Epic, Oracle Health, Dedalus, and NEXUS are also repositioning around cloud, interoperability, and AI, often with simpler product architectures or stronger acute care concentration.

4

Competitor Profiles

4

Competitor Profiles

This section profiles the vendors that most directly shape CompuGroup Medical’s strategic room for maneuver in 2026. The competitive set is bifurcated between U.S. scale leaders that dominate acute care and ambulatory mindshare, and European incumbents that matter more through regional procurement density, regulatory fit, and consolidation optionality than through global brand power.^{29 100}

For investors, the key distinction is not simply who is largest, but which competitors combine installed-base control, customer satisfaction, cloud migration credibility, and AI monetization pathways. Epic remains the benchmark for acute care execution, Oracle Health is still strategically relevant despite share loss because of Oracle’s capital base and platform ambition, and athenahealth, eClinicalWorks, and MEDITECH each retain defensible positions in ambulatory, community, and mid-market segments that overlap with parts of CGM’s addressable opportunity.^{26 175 156}

4.1

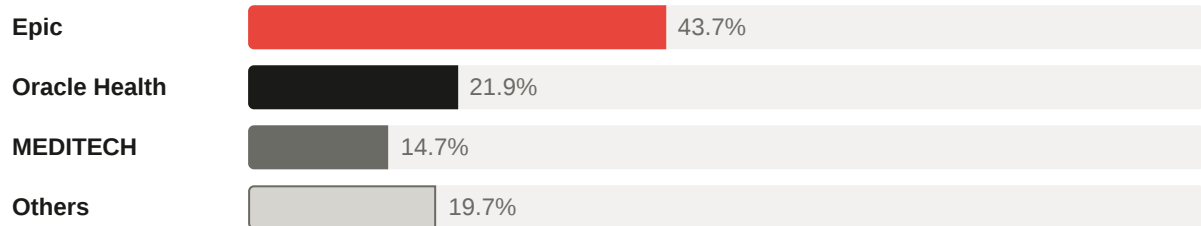
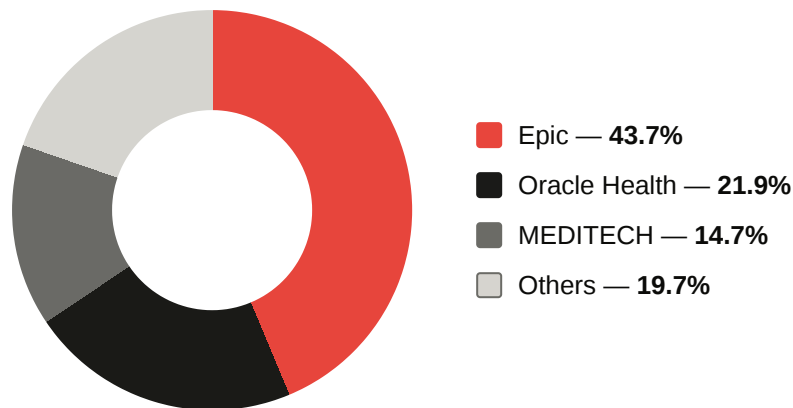
Epic Systems: Market Dominance, Financials, and Global Growth Strategy

Epic is the clearest scale and quality benchmark in provider IT. KLAS reported that Epic held 43.7 percent of the U.S. acute care hospital EHR market in 2025 among multispecialty acute care hospitals, versus 21.9 percent for Oracle Health and 14.7 percent for MEDITECH, and industry coverage indicates Epic now supports more than half of U.S. acute multispecialty hospital beds.^{117 119 148}

Title — U.S. acute care EHR share snapshot 2025

VENDOR	HOSPITAL SHARE %	STRATEGIC IMPLICATION
Epic	43.7	Category leader with strongest replacement momentum and referenceability.
Oracle Health	21.9	Still scaled, but under pressure from attrition and platform transition.
MEDITECH	14.7	Durable community and mid-market base with strong retention.
Others	19.7	Fragmented tail with limited national influence.

Source: ^{117 29}



Epic's financial profile remains unusually strong for a private healthcare software company, though disclosure is limited. Forbes reported Epic revenue of about USD 4.6 billion in 2022, while MedCity News cited 2024 revenue of USD 5.7 billion, implying continued double-digit scale expansion and substantial self-funded R&D capacity without public market pressure.¹⁴⁴

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- Market dominance: Epic won the 2026 Overall Best in KLAS award for Health System Suites, reinforcing that its share leadership is paired with superior customer perception rather than being a legacy installed-base artifact.^{175 26}
- Strategic model: Epic's advantage is vertical integration across clinical, revenue cycle, patient engagement, interoperability, and increasingly AI-enabled workflow tools, which raises switching costs and compresses the room for subscale vendors in large hospital tenders.^{144 29}
- Global growth: KLAS's 2024 global acute care market share commentary identified Epic as the winner of the largest validated 2023 international decision and showed continued selection momentum outside the U.S., indicating that Epic's international strategy is no longer opportunistic but increasingly systematic.¹²²
- AI posture: Epic has been an early commercializer of generative AI use cases in coding, summarization, and workflow support, often through partnerships rather than greenfield model development. That matters for CGM because it raises customer expectations for embedded AI inside core workflows, not as a bolt-on module.¹⁴⁴
- Read-through for CGM: Epic is not the most direct competitor in German ambulatory or pharmacy software, but it is the strategic ceiling for hospital platform quality, implementation credibility, and AI-enabled suite economics. Any CGM hospital expansion thesis in Western Europe must assume Epic remains the premium reference vendor for large-system modernization.^{122 100}

4.2

Oracle Health (Cerner): Market Position, Customer Attrition, and AI Platform Pivot

Oracle Health remains one of the few vendors with enough installed base, capital, and technical ambition to matter strategically despite deteriorating customer sentiment. KLAS reported that Oracle Health was one of only five vendors with validated net-new acute care decisions in 2025, but separate KLAS analysis and secondary coverage indicate the company has lost meaningful market share and customer confidence in the three years since Oracle acquired Cerner.^{29 113 115 116}

Oracle's current position is therefore paradoxical. It still controls 21.9 percent of U.S. acute care hospitals and 20.4 percent of beds according to 2025 KLAS-based reporting, yet that scale now functions more as a restructuring platform than as evidence of competitive momentum.^{117 121}

- Attrition and satisfaction: KLAS's 2025 Oracle Health report examined customer interviews from 2022 through 2025 and found that Oracle had not materially improved customer experience post-acquisition. Secondary summaries describe declining relationships, communication issues, and dissatisfaction with execution, which are classic precursors to replacement cycles in high-switching-cost software markets.^{113 115 116}
- Market position: Oracle Health remains the number two acute care EHR vendor by U.S. hospital share, which means it still has strategic relevance in every large replacement discussion even when it is not the favored bidder.^{117 29}
- Platform pivot: Oracle is attempting to reset the narrative through a cloud-native rebuild and AI-led product positioning rather than incremental modernization of Cerner Millennium. Industry reporting in 2026 indicates Oracle plans to expand its new OCI-based EHR architecture into acute care, making 2026 to 2027 a proving period for whether technical reinvention can offset installed-base erosion.^{121 114}
- AI strategy: Oracle Health has emphasized Clinical AI Agent and broader AI-enabled workflow tooling. The strategic logic is sound because Oracle can combine infrastructure, database, and application layers, but customer willingness to adopt depends on restoring trust in core delivery first.¹¹⁴
- Organizational risk: Bloomberg reported in March 2026 that Oracle's health unit had lost multiple senior leaders while trying to modernize the platform, which increases execution risk during an already delicate transition.¹²³
- Read-through for CGM: Oracle Health is a weaker near-term execution benchmark than Epic, but it remains a credible long-term threat because Oracle can subsidize product redevelopment and pursue aggressive AI and cloud bundling. For CGM, Oracle's instability creates tactical openings in hospital replacement cycles, especially in Europe, but those openings may narrow if Oracle's platform reset gains traction after 2026.^{113 100}

4.3

Athenahealth, eClinicalWorks, and Meditech: Ambulatory and Niche Positioning

Below the Epic and Oracle tier, athenahealth, eClinicalWorks, and MEDITECH occupy durable positions that matter to CGM because they define the economics of ambulatory, community, and mid-market provider IT. None matches Epic's acute care dominance, but each controls a

segment where workflow fit, customer retention, and recurring revenue quality can be highly attractive.^{156 135 158}

athenahealth is the strongest ambulatory benchmark in this group. The company won five 2026 Best in KLAS awards, including Overall Independent Physician Practice Suite for the third consecutive year, and is positioning athenaOne as an AI-native, network-enabled operating layer for independent and enterprise ambulatory groups.^{156 139 175}

- Athenahealth: Its strategic strength is the combination of cloud delivery, embedded RCM services, and a large connected network, which makes it more than a software vendor and supports premium retention in physician practices. June 2026 company disclosures highlighted more than 80 AI-native revenue cycle features, underscoring that athenahealth is using automation to defend both software ARPU and service economics.^{139 146 156}
- eClinicalWorks: eClinicalWorks remains a scaled private ambulatory vendor with 2024 revenue of USD 900 million and 6,500 employees according to its company overview. Its positioning is strongest in cost-conscious ambulatory, multispecialty, and community settings where breadth, integrated PM and RCM, and large installed-base familiarity matter more than premium enterprise branding.^{135 138}
- eClinicalWorks product posture: The company continues to emphasize AI documentation, patient engagement, and interoperability through PRISMA, which it says exchanges 2.6 million patient records per day. That suggests a strategy centered on practical workflow utility rather than a full-suite hospital platform play.¹³⁸
- MEDITECH: MEDITECH remains the most defensible non-Epic acute care niche vendor in the U.S. community and mid-market hospital segment. KLAS-based 2025 market share reporting placed it at 14.7 percent of acute care hospitals, and the company highlighted a 12th consecutive Best in KLAS recognition in 2026 for small acute care EHR and patient accounting.^{117 158}
- MEDITECH strategic role: Its core advantage is not aggressive share capture but high retention, community-hospital fit, and a credible Expanse modernization path. That makes MEDITECH relevant to CGM as a model for how a focused vendor can preserve value in subscale acute care segments without matching Epic's breadth.^{119 158}
- European read-through: These three vendors are less directly threatening to CGM in DACH hospital core systems than Epic, Oracle, Dedalus, or NEXUS, but they shape buyer expectations around cloud cadence, ambulatory usability, and recurring revenue models. athenahealth is the most relevant comparator for CGM's ambulatory cloud transition, while MEDITECH is the better analogue for defending a sticky but narrower hospital niche.^{100 156 158}
- Scope note: Reliable up-to-date public financial disclosure remains limited for private vendors such as athenahealth and MEDITECH, so this profile relies more heavily on official positioning, KLAS recognition, and market-share evidence than on audited margin or valuation data.^{156 158}

5

Head-to-Head Benchmarking: Key Performance Metrics

5

Head-to-Head Benchmarking: Key Performance Metrics

This section benchmarks CGM against the most relevant global and European health IT vendors on the metrics that matter most for investors and M&A practitioners: installed-base share, revenue scale, margin quality, recurring revenue visibility, and innovation intensity. The central conclusion is that CGM remains subscale versus U.S. acute care leaders and the largest European hospital platforms, but it compares more favorably on revenue visibility and breadth across ambulatory, hospital, and pharmacy workflows than many regional peers, while still carrying a heavier modernization burden than more focused competitors.^{100 29 185}

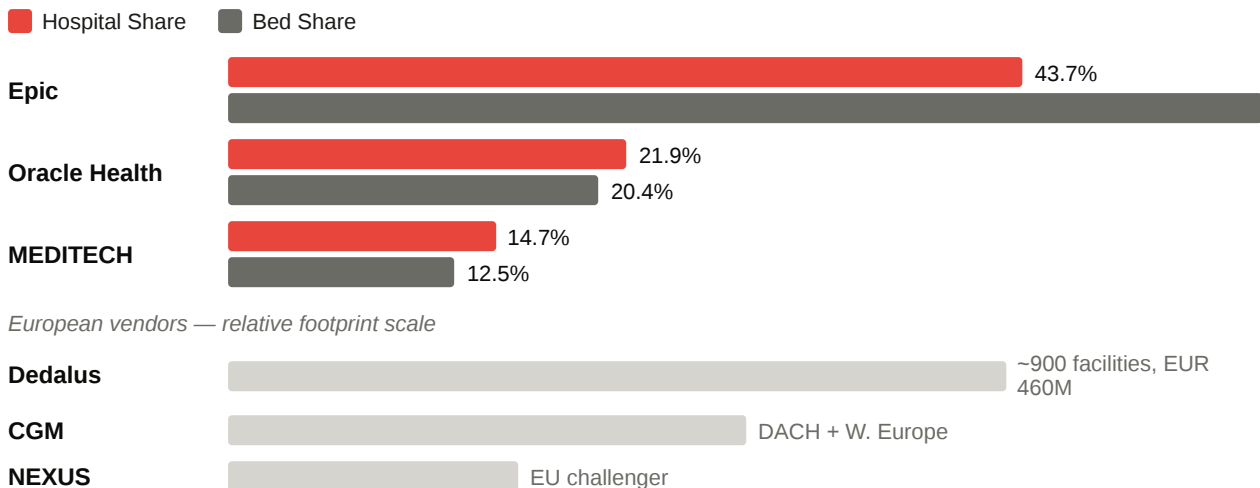
5.1

Market Share Comparison

Title — Selected vendor scale and market share indicators

VENDOR	SHARE INDICATOR	STRATEGIC READ-THROUGH
Epic	43.7% of U.S. acute care hospitals; 56.9% of U.S. acute care beds in 2025.	Clear scale leader in large-system acute care, with the strongest reference position in replacement cycles.
Oracle Health	21.9% of U.S. acute care hospitals; 20.4% of beds in 2025.	Still the number two acute care platform by installed base, but increasingly a restructuring asset rather than a momentum story.
MEDITECH	14.7% of U.S. acute care hospitals; 12.5% of beds in 2025.	Durable community and mid-market acute care niche with strong retention economics.
CGM	Material DACH and Western Europe ambulatory, hospital, and pharmacy footprint; KLAS identified at least one non-US hospital migration from CGM KIMSYSTEM to CGM REHA in the 2024 global market-share cycle.	Relevant regional incumbent, but not a top-tier global acute care share holder.
Dedalus	KLAS identified Dedalus as adding the most beds in the 2024 non-US acute care market among measured vendors; in DACH, Dedalus reported about 900 facilities using Orbis and 2025 DACH revenue of about EUR 460 million.	Largest hospital-centric European rival and the most important regional comparator for hospital procurement density.
NEXUS	Strong European hospital software footprint, but materially smaller than CGM and Dedalus by revenue scale.	Important regional challenger, especially in German-speaking and adjacent European hospital markets.

Source: 117 183 185 230



U.S. figures: 2025 acute care hospital and bed share. European vendors shown as relative footprint estimates only (no published U.S. share data).

- Acute care concentration: U.S. hospital share remains highly concentrated, with Epic, Oracle Health, and MEDITECH together accounting for roughly 80.3% of acute care hospitals and 89.8% of beds in 2025. That concentration sets the global benchmark for scale, implementation credibility, and data-network effects, and it underscores how far CGM is from the top tier in acute care outside its home markets.^{117 183}
- Global hospital positioning: KLAS's non-U.S. market-share work indicates Epic had the largest global EHR market share among measured vendors, while Dedalus added the most beds in the 2024 non-U.S. cycle. That matters for CGM because it implies the company is squeezed between a premium global acute care leader and a scaled European hospital consolidator.^{185 187}
- DACH hospital footprint: Dedalus stated that 850 German facilities and about 900 DACH facilities were using Orbis in 2025, reinforcing its status as the hospital market leader in Germany. By contrast, CGM's hospital relevance is meaningful but more selective, and its strategic value lies more in cross-setting breadth than in outright hospital share leadership.^{230 100}
- Ambulatory footprint: In U.S. clinician markets, federal rulemaking commentary citing 2024 and 2025 industry reporting states that Epic, Oracle Cerner, and athenahealth together represent about 82% of clinician market share. CGM's ambulatory strength is therefore much more regional than global, with its defensibility concentrated in Germany and selected Western European markets rather than in the U.S. physician office mainstream.^{192 100}
- Investor read-through: CGM's market-share profile is best understood as dense regional entrenchment rather than global category leadership. That supports strategic value in DACH and Western Europe, but it also limits multiple expansion unless management can convert local share into higher-growth cloud, AI, and workflow monetization.^{100 185}

5.2

Revenue, EBITDA Margins, and Recurring Revenue Ratios

- CGM baseline: CGM reported FY2025 revenue of EUR 1.213 billion, adjusted EBITDA of EUR 240 million, and an adjusted EBITDA margin of 16%, with recurring revenue of EUR 880.5 million or about 73% of sales. For investors, that places CGM below the largest U.S. and European acute care platforms on scale, but above many regional peers on disclosed revenue visibility and recurrence.¹⁰⁰

- NEXUS comparison: NEXUS generated EUR 261.5 million of 2024 revenue and EUR 57.6 million of EBITDA, implying an EBITDA margin of about 22.0%. Its software maintenance revenue was EUR 140.6 million, equal to roughly 53.8% of sales, which is lower than CGM's recurring mix but still indicative of a sticky maintenance-led model.^{211 216}
- Epic scale: Forbes reported Epic had USD 4.9 billion of annual revenue in 2024, up from about USD 4.6 billion in 2022. Epic does not publicly disclose EBITDA or recurring revenue ratios, but its private ownership, large installed base, and broad suite economics strongly imply superior cash generation and reinvestment capacity relative to CGM.^{239 144}
- Oracle scale and margin context: Oracle reported total fiscal 2026 revenue of USD 57.4 billion and continues to absorb Oracle Health within broader cloud and applications reporting, so a clean Oracle Health EBITDA benchmark is not publicly disclosed. For benchmarking purposes, Oracle Health benefits from parent-level capital and infrastructure scale, but segment opacity reduces comparability for standalone M&A-style analysis.^{232 235}
- eClinicalWorks benchmark: eClinicalWorks states that it was consistently profitable and generated USD 900 million of revenue in 2024. The absence of audited public EBITDA disclosure limits precision, but the combination of cloud delivery, ambulatory concentration, and profitability suggests a margin profile that is likely structurally healthier than hospital-heavy legacy vendors, though not directly comparable to CGM's multi-segment mix.¹³⁵
- Dedalus benchmark: Dedalus remains financially opaque as a private sponsor-backed company, but S&P Global Ratings cited about EUR 771 million of pro forma 2021 revenue and EUR 116 million of S&P-adjusted EBITDA, while Ardian said the DXC healthcare software acquisition would bring the combined business to around EUR 700 million of turnover with about 2,000 R&D staff. More recent operating commentary from Dedalus DACH points to about EUR 460 million of 2025 revenue in DACH alone, implying that group scale remains at least comparable to, and likely above, CGM in hospital-centric Europe.^{234 229 230}
- Revenue quality conclusion: CGM's 73% recurring revenue ratio is one of the strongest disclosed metrics in the peer set and is a meaningful valuation support. The trade-off is that its 16% adjusted EBITDA margin in 2025 sits below NEXUS's 2024 EBITDA margin and below what investors would typically infer for highly scaled private leaders such as Epic, reflecting CGM's broader portfolio complexity and ongoing transformation costs.^{100 211 239}

5.3

R&D Investment Intensity and Innovation Spend as a Proportion of Revenue

- CGM intensity: CGM reported EUR 274 million of R&D expense in 2025 on EUR 1.213 billion of revenue, equal to roughly 22.6% of sales, up from EUR 255 million in 2024. That is a high reinvestment rate for a mature health IT vendor and signals that management is funding a broad modernization agenda across ambulatory, hospital, pharmacy, cloud, and AI layers simultaneously.^{100 17}
- NEXUS intensity: NEXUS does not present R&D in the same headline format as CGM, but its 2024 annual report shows capitalized development costs and a software-heavy operating model, while total investments in intangible assets and property, plant and equipment were EUR 10.4 million. On disclosed data, NEXUS appears materially less R&D-intensive than CGM, which is consistent with its narrower scope and more focused hospital software footprint.²¹¹
- Dedalus intensity: Ardian stated in 2020 that the enlarged Dedalus would employ over 5,500 people, including about 2,000 solely in R&D, and described the company as creating the world's largest R&D platform for the sector. Even though current group R&D expense is not publicly

disclosed, that staffing profile indicates innovation spend at a scale that likely exceeds CGM in hospital-centric software, especially when measured by absolute headcount.²²⁹

- Epic and Oracle: Neither Epic nor Oracle Health discloses a clean health-IT-only R&D ratio. Oracle's consolidated R&D is substantial at the parent level, but it is not attributable specifically to Oracle Health, while Epic's private status prevents precise benchmarking despite clear evidence of heavy product reinvestment in AI, interoperability, and acute care workflows.^{232 239}
- eClinicalWorks and athenahealth: Publicly reliable, current R&D ratios were not located for either company in this search. That absence itself is analytically relevant, because it means CGM offers investors more transparency on innovation intensity than several important private peers, even if those peers may be investing aggressively in AI and cloud productization.^{135 156}
- Investor conclusion: CGM's R&D burden is both a strength and a valuation constraint. It supports the thesis that the company is actively rebuilding its platform economics, but at more than one-fifth of revenue it also highlights how much capital is required to modernize a broad legacy estate, which helps explain why CGM's margin profile trails more focused or more scaled peers despite strong recurring revenue quality.^{100 229 211}

6

Product and Capability Benchmarking

6

Product and Capability Benchmarking

This section benchmarks CGM's product stack against major U.S. and European health IT vendors on the dimensions that increasingly determine strategic value creation in 2026: breadth of core EHR and HIS functionality, cloud delivery maturity, interoperability architecture, and the ability to operationalize AI and analytics inside daily workflows. The investor conclusion is that CGM remains credible and increasingly modern in ambulatory, pharmacy, and selected hospital workflows, but it still trails the top U.S. platforms and the strongest hospital-centric peers in platform unification, cloud operating maturity, and scaled AI commercialization.^{100 175 282}

The competitive pattern is bifurcated. Epic, athenahealth, and increasingly MEDITECH present more coherent product narratives around integrated workflows and continuous delivery, while Oracle Health is attempting a high-risk but potentially powerful reset around OCI and Clinical AI Agent; European rivals such as Dedalus and NEXUS remain relevant because local workflow depth and regulatory fit still matter in DACH and Western Europe, even when their public product disclosure is less granular than U.S. peers.^{263 255 211}

6.1

EHR/HIS Feature Comparison across Vendors

Title — Core product capability positioning by vendor

VENDOR	CORE PRODUCT STRENGTHS	RELATIVE GAPS VERSUS LEADERS
CGM	Broad cross-setting footprint across ambulatory, hospital, pharmacy, and connectivity; CGM Praxis on CGM ONE for ambulatory cloud workflows; CGM CLINICAL Suite and CGM CLINICAL RCM for hospital modernization.	Product breadth is an advantage, but portfolio complexity and uneven modernization reduce the simplicity and referenceability seen at Epic or athenahealth.
Epic	Deeply integrated acute and ambulatory suite spanning clinical, revenue cycle, patient engagement, and interoperability; strongest enterprise workflow coherence and customer validation.	Less directly exposed to European pharmacy and country-specific ambulatory niches where CGM has local density.
Oracle Health	Large acute-care feature set, unified patient record ambition, and broad clinical plus financial workflow coverage; still relevant in enterprise tenders due to installed base.	Customer dissatisfaction and transition risk weaken confidence in current execution and reduce product credibility relative to Epic.
athenahealth	Highly integrated ambulatory EHR, PM, RCM, and patient engagement stack with strong usability and cloud operating model.	Less competitive in full acute-care HIS and large hospital command-center workflows.
eClinicalWorks	Broad ambulatory feature set with integrated PM, telehealth, patient engagement, and interoperability tools such as PRISMA and healow.	Enterprise hospital depth and premium large-system functionality remain limited relative to Epic, Oracle, and MEDITECH.

VENDOR	CORE PRODUCT STRENGTHS	RELATIVE GAPS VERSUS LEADERS
MEDITECH	Strong community and mid-market acute-care EHR with unified revenue cycle, interoperability, and increasingly AI-enabled workflow support through Expanse.	Narrower global breadth and lower large-academic-system prestige than Epic.
Dedalus	Strong hospital-centric footprint in Europe, especially through Orbis and regional workflow localization.	Public evidence on product modernization detail is less transparent, making direct capability benchmarking less precise.
NEXUS	Focused hospital software and diagnostic workflow depth in Europe with sticky installed-base characteristics.	Smaller scale and narrower suite breadth than CGM, Dedalus, or U.S. enterprise leaders.

Source: ^{100 263 282 280}

CGM's feature proposition is strongest where buyers value cross-setting coverage rather than best-of-breed dominance in a single care setting. Its ambulatory, hospital, and pharmacy assets create more cross-sell surface area than most peers, but that same breadth raises implementation and product harmonization demands, which matters in procurement processes increasingly biased toward cleaner platform stories. ^{100 92}

- CGM: The company's current product roadmap centers on CGM Praxis as a cloud ambulatory platform, CGM ONE workflow add-ons, and CGM CLINICAL Suite plus CGM CLINICAL RCM in hospitals. For investors, this indicates a deliberate move from fragmented legacy modules toward a more monetizable platform model, but not yet a fully unified suite on the level of Epic's enterprise architecture. ^{100 92}
- Epic: Epic remains the benchmark for end-to-end feature completeness in large health systems, combining clinical documentation, patient access, revenue cycle, interoperability, and embedded AI within a single operating environment. Its 2026 Overall Best in KLAS award for Health System Suites reinforces that product breadth is matched by customer execution credibility. ^{175 144}
- Oracle Health: Oracle still offers broad acute-care functionality and a unified-record vision, but the product is in transition. The strategic issue is not missing modules so much as whether the installed base trusts Oracle to modernize and deliver them effectively after several years of weak customer sentiment. ^{255 113 116}
- athenahealth and eClinicalWorks: Both are stronger than CGM in the coherence of ambulatory workflow packaging, especially where EHR, PM, RCM, patient engagement, and cloud delivery are evaluated as one operating stack. athenahealth is the premium benchmark in this segment, while eClinicalWorks competes effectively on breadth and cost efficiency, supported by PRISMA-based interoperability and healow patient-facing tools. ^{263 156 280}
- MEDITECH, Dedalus, and NEXUS: MEDITECH is the clearest hospital product comparator for a focused but modern acute-care platform, while Dedalus and NEXUS matter more in Europe because of local deployment density and workflow fit. Dedalus appears stronger than CGM in hospital procurement gravity, whereas NEXUS is narrower but often operationally focused in specialty and hospital-adjacent workflows. ^{282 273 211}

The practical investor takeaway is that CGM is not feature-deficient in absolute terms. Its challenge is comparative product coherence: buyers increasingly reward vendors whose EHR,

revenue cycle, patient engagement, interoperability, and AI layers feel like one continuously updated platform rather than a federation of strong but uneven assets.^{100 263 282}

6.2

Cloud Readiness and Platform Integration Maturity

Cloud readiness is now a proxy for more than hosting architecture. It determines release cadence, integration economics, AI deployment speed, and the ability to convert maintenance-heavy installed bases into higher-quality subscription and platform revenue, which is why this dimension has direct valuation relevance.^{100 251 250}

- CGM: CGM has made visible progress through CGM ONE, CGM Praxis, cloud pharmacy initiatives such as CGM STELLA, and open CGM G3 technology in hospitals. Its C5 attestation in Germany is commercially important because security assurance is a gating factor for regulated cloud adoption, but the company is still in migration mode rather than operating from a fully cloud-native installed base.^{100 271}
- Epic: Epic's cloud posture is more hybrid than athenahealth's, but its integration maturity is exceptionally high because the suite is already operationally unified and increasingly connected to external cloud and AI ecosystems. In practice, Epic's advantage is not pure cloud nativeness but the ability to embed new capabilities into a highly standardized enterprise workflow fabric.^{175 144}
- Oracle Health: Oracle's strategic thesis is the most explicitly cloud-centric in the peer set because it is rebuilding around Oracle Cloud Infrastructure and a unified data and workflow layer. If successful, that could materially improve Oracle's competitive position, but as of mid-2026 it remains a transition story with elevated execution risk rather than a proven operating advantage.^{260 255 121}
- athenahealth: athenahealth remains the cleanest cloud-native benchmark in ambulatory care. Its integrated EHR, PM, RCM, and patient engagement stack is delivered as a continuously updated platform, which lowers upgrade friction and supports rapid rollout of AI-native features across a large network.^{263 256 259}
- eClinicalWorks: eClinicalWorks is also structurally advantaged in cloud ambulatory delivery and has built meaningful interoperability rails through PRISMA and the healow FHIR developer ecosystem. Its weakness is not cloud readiness but limited relevance in enterprise hospital platform decisions, where integration complexity is materially higher.^{280 272}
- MEDITECH: MEDITECH has improved materially through Expanse and MEDITECH as a Service, positioning a secure cloud-native platform with an open ecosystem and broad interoperability partnerships. For CGM, MEDITECH is a useful proof point that a legacy acute-care vendor can modernize credibly without abandoning its installed base economics.^{282 274 276}
- Dedalus and NEXUS: Both remain strategically relevant in Europe, but public evidence suggests less transparent cloud messaging and fewer globally visible platform artifacts than the leading U.S. peers. That does not imply weak products, but it does imply lower external proof of integration maturity, which can matter in cross-border M&A diligence and investor perception.^{211 230}

Academic and technical literature reinforces why this matters. Recent work on FHIR transformation and cloud-native clinical analytics shows that scalable interoperability increasingly depends on reusable mappings, event-driven architectures, and analytics layers that can support real-time model serving, not just static interface engines; by that standard,

CGM is directionally aligned but still earlier in platform industrialization than the strongest cloud-native peers.^{250 253 251}

The strategic ranking is therefore clear. athenahealth leads in ambulatory cloud operating maturity, Epic leads in integrated enterprise workflow maturity, MEDITECH is a credible hospital modernization case, Oracle has the highest upside but also the highest transition risk, and CGM sits in the middle with credible progress but a larger installed-base migration burden than the market leaders.^{263 175 282 255 100}

6.3

AI and Analytics Capabilities: Where Each Vendor Stands Relative to CGM

AI capability in provider IT is no longer judged by model novelty alone. Buyers increasingly care about whether vendors can embed summarization, ambient documentation, chart review, coding support, search, and operational analytics directly into the EHR without creating governance, workflow, or interoperability friction, a pattern also emphasized in recent healthcare AI literature.^{44 247}

CGM's AI posture is credible and commercially relevant, but still emerging relative to the leaders. The company has invested in AI, machine learning, and large language models since 2023 and has productized use cases such as CGM AMBI in the U.S., DokuAssistant, CGM ONE telephone assistant, and CGM HIS Assistant, which positions it well in documentation and administrative automation but not yet as a category setter in enterprise-scale clinical AI.^{100 93 92}

- Relative to Epic: Epic remains ahead of CGM in scaled AI commercialization because it is embedding generative AI into a deeply integrated enterprise suite and benefits from stronger large-system data density and workflow control. For CGM, the implication is that AI parity in isolated features will not be enough; it needs tighter platform integration and broader deployment evidence.^{144 175}
- Relative to Oracle Health: Oracle's Clinical AI Agent is more ambitious than CGM's current AI stack in scope, spanning chart summarization, note drafting, coding suggestions, workflow orchestration, and connected clinical, operational, and financial data. However, Oracle's advantage is still partly aspirational because customer trust and platform transition remain unresolved, which narrows the practical gap versus CGM in the near term.^{255 257}
- Relative to athenahealth: athenahealth is ahead of CGM in ambulatory AI industrialization. By June 2026 it had launched more than 80 AI-native revenue cycle features, and by August 13, 2026 it said AI-native athenaOne experiences were available to more than 170,000 clinicians, giving it stronger evidence of scaled deployment and measurable workflow penetration than CGM currently discloses.^{258 259}
- Relative to eClinicalWorks: eClinicalWorks is using AI pragmatically in interoperability search, contact-center automation, and ambulatory workflow compression. Its AI Assistant for PRISMA and healow Genie show a narrower but commercially tangible approach that in some ambulatory use cases is at least as mature as CGM's current AI packaging.^{272 280}
- Relative to MEDITECH: MEDITECH is increasingly competitive in AI-enabled acute-care workflows through ambient intelligence, denial management, Expanse Navigator, and AI-ranked search across internal and external data. This places MEDITECH ahead of CGM in the clarity of hospital AI narrative, even if CGM retains broader exposure across non-hospital settings.^{283 275}

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- Relative to Dedalus and NEXUS: Publicly available evidence is less detailed, so any ranking must be more cautious. The available record supports the inference that both remain relevant in analytics and workflow digitization within European hospital environments, but neither currently presents as visibly scaled in AI commercialization as Epic, Oracle, athenahealth, or MEDITECH.^{211 230}

From an analytics perspective, CGM's opportunity is substantial because its cross-setting footprint could support richer longitudinal data products than many single-setting peers. The constraint is execution: recent research emphasizes that healthcare AI value depends on data quality, multimodal retrieval, semantic interoperability, and trustworthy workflow integration, and those are precisely the areas where CGM still needs to prove scaled operational maturity rather than isolated feature innovation.^{44 247 250}

The investor-grade conclusion is that CGM is no longer absent from the AI race, but it is not yet setting the pace. Its most realistic path to value creation is to use AI to raise ARPU, retention, and workflow stickiness inside its installed base, especially in German ambulatory, pharmacy, and selected hospital modernization programs, rather than trying to outspend Epic or Oracle in enterprise-scale AI platform ambition.^{100 92}

7

Regional Competitive Analysis

7

Regional Competitive Analysis

Regional competitive dynamics are central to CGM’s investment case because the company’s economic moat is geographically uneven. In DACH and Western Europe, CGM benefits from regulatory localization, ambulatory and pharmacy workflow entrenchment, and a broad installed base, while in the United States it competes from a narrower ambulatory and specialty position against vendors with materially greater scale, KLAS visibility, and enterprise referencability such as Epic, Oracle Health, athenahealth, eClinicalWorks, and MEDITECH.^{100 185 26}

For investors and M&A professionals, the key question is not whether CGM can be globally dominant, but whether it can defend high-value regional strongholds while selectively expanding into adjacencies where incumbents are weaker or local complexity is high. The regional pattern suggests that CGM’s best risk-adjusted growth path remains installed-base monetization in Europe, disciplined niche expansion in North America, and targeted international growth in markets where hospital and ambulatory digitization are still fragmented enough to reward localized platforms rather than winner-take-all mega suites.^{17 311 184}

7.1

DACH and Western Europe: CGM's Home Turf vs. Encroaching Global Rivals

CGM’s home-market advantage remains substantial because its portfolio is aligned with the fragmented provider structure of Germany and adjacent Western European markets, where ambulatory practices, pharmacies, and mid-sized hospitals still value local workflow fit, reimbursement logic, and compliance tooling. In FY2025, Germany generated EUR 684.5 million of CGM’s third-party revenue and Western Europe generated EUR 356.1 million, meaning roughly 86 percent of group revenue still came from those two regions combined.¹⁰⁰

Title — Western Europe competitive positioning snapshot

VENDOR	REGIONAL EVIDENCE	STRATEGIC IMPLICATION
CGM	Germany revenue EUR 684.5m in 2025; Western Europe revenue EUR 356.1m.	Strongest where ambulatory, pharmacy, and localized compliance workflows matter most.
Dedalus	About EUR 460m DACH revenue in 2025; 850 German facilities and 900 DACH facilities using Orbis.	Largest hospital-centric regional rival with superior procurement gravity in acute care.
NEXUS	EUR 261.5m revenue and EUR 57.6m EBITDA in 2024.	Smaller than CGM and Dedalus, but a credible focused challenger in hospital software.
Epic	Largest global EHR market share among measured vendors in KLAS non-US analysis.	Premium threat in large-system hospital modernization, especially cross-border tenders.

Source: ^{100 230 216 185}

- Home-turf strength: CGM’s defensibility is strongest in ambulatory and pharmacy software, where country-specific regulation and workflow complexity create higher switching friction than

generic software categories. This is reinforced by the company's broad recurring base and by its role in German digital infrastructure layers such as e-prescription and practice connectivity.¹⁷

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- Hospital pressure point: Dedalus is the most immediate structural threat in DACH hospitals. Its disclosed 2025 DACH revenue of about EUR 460 million and Orbis footprint of 850 German facilities and 900 DACH facilities indicate greater acute-care density than CGM, which gives Dedalus stronger referencability in large hospital tenders and modernization cycles.²³⁰
- Global encroachment: Epic is not yet a mass-market DACH ambulatory competitor, but it is increasingly relevant in Western European flagship hospital procurements. KLAS reported Epic as the largest global EHR vendor among measured non-US vendors, while Dedalus added the most beds outside the U.S., underscoring that CGM faces pressure from both a premium global suite vendor and a scaled European hospital consolidator.^{185 184}
- Secondary regional challengers: NEXUS remains strategically relevant because it combines a smaller revenue base with solid profitability and sticky hospital workflows. Its 2024 revenue of EUR 261.5 million and EBITDA of EUR 57.6 million imply a focused competitor that can defend specialty and hospital-adjacent niches even if it lacks CGM's cross-setting breadth.²¹⁶
- Investor read-through: In DACH and Western Europe, CGM is best viewed as a defensible regional platform rather than the outright hospital share leader. The valuation upside comes from converting local density into higher-ARPU cloud, AI, and workflow services before Dedalus deepens hospital control and before Epic captures more premium cross-border reference sites.^{100 185}

7.2

United States: CGM's Niche Positioning vs. Epic, Oracle, and Athenahealth Dominance

CGM's U.S. position is strategically relevant but clearly niche relative to the dominant platforms. North America contributed EUR 133.4 million of CGM's FY2025 revenue, which is meaningful as a diversification vector but small compared with the scale and market power of Epic, Oracle Health, athenahealth, eClinicalWorks, and MEDITECH in their core U.S. segments.¹⁰⁰

The competitive challenge is structural. Epic remains the top overall health system suite in the 2026 Best in KLAS awards, athenahealth repeated as the top overall independent physician practice suite for the third consecutive year, and eClinicalWorks continues to operate at substantial ambulatory scale with stated 2024 revenue of USD 900 million, leaving limited whitespace for a subscale entrant without a sharply differentiated workflow proposition.^{157 156}

135

- Market structure: U.S. acute care remains highly concentrated around Epic, Oracle Health, and MEDITECH, while ambulatory mindshare is led by Epic and athenahealth in the independent suite segment. This concentration raises customer acquisition costs and makes broad-based share capture unrealistic for CGM without acquisition or a highly differentiated specialty strategy.^{26 314}
- CGM's likely lane: CGM's U.S. opportunity is more plausibly in ambulatory, specialty, lab-adjacent, and AI-enabled documentation workflows than in enterprise acute care displacement. The company's U.S. product messaging around CGM AMBI and adjacent ambulatory solutions suggests a strategy of workflow augmentation rather than direct confrontation with Epic or Oracle in large hospital core systems.¹⁰⁰

- Competitive benchmark: athenahealth is the most relevant U.S. comparator for CGM's ambulatory ambitions because it combines cloud delivery, RCM integration, and strong KLAS recognition. Its June 3, 2026 disclosure of more than 80 AI-native revenue cycle features shows how far the leading ambulatory platforms have moved beyond core EHR into automation-led operating leverage.^{146 156}
- Regulatory burden: U.S. interoperability requirements continue to raise the bar for all vendors. ONC states that HTI-1 adopts USCDI Version 3 as the new baseline standard in the certification program as of January 1, 2026, which favors vendors with larger compliance budgets and mature API ecosystems.³¹¹
- Investor read-through: CGM's U.S. business should be valued as an option on focused ambulatory and AI workflow growth, not as a credible near-term challenger to the dominant acute-care stack. The strategic merit lies in selective niches that can scale without requiring head-on competition against Epic's enterprise dominance, Oracle's installed base, or athenahealth's ambulatory brand strength.^{100 157}

7.3

Emerging Markets and International Expansion Trajectories

CGM's international expansion logic is more selective than that of the largest global EHR vendors. The company reported offices in 19 countries and product distribution in 60 countries, but FY2025 revenue outside Germany, Western Europe, North America, and Eastern Europe remained very small, with only EUR 5.5 million reported in the rest-of-world category, indicating that international optionality exists but is not yet a material earnings driver.^{100 17}

The opportunity set is nevertheless real because many non-U.S. markets remain less consolidated than the American acute-care market and still reward localized deployment models. KLAS's global non-U.S. market-share work shows a broad field of regional decisions and migrations across Europe, the Middle East, Asia-Pacific, and Latin America, which implies that sub-global vendors can still win where procurement is fragmented and local adaptation matters.¹⁸⁵

- Most plausible expansion vector: Eastern Europe and selected adjacent international markets are more realistic for CGM than greenfield entry into highly consolidated Anglo markets. CGM already generated EUR 33.5 million of FY2025 revenue in Eastern Europe, providing a modest but existing platform from which to extend ambulatory, hospital, and pharmacy offerings.¹⁰⁰
- Why emerging markets matter: In less mature provider IT markets, buyers often prioritize localization, implementation flexibility, and total cost of ownership over the prestige of a global mega-suite. That dynamic can favor CGM's modular portfolio, especially where ambulatory and pharmacy digitization are earlier in the adoption curve and where hospital replacement cycles are not yet locked into Epic-style enterprise standards.¹⁸⁵
- Constraint set: The same factors that create opportunity also create execution risk. Emerging markets often require local partnerships, country-specific compliance work, and lower price points, which can dilute margins unless CGM exports standardized cloud products rather than replicating its historically fragmented on-premises model.¹⁰⁰
- Competitive context: Dedalus and Epic are also expanding internationally, with KLAS identifying Dedalus as the vendor adding the most beds outside the U.S. in the 2024 cycle and Epic as the largest global EHR vendor among measured vendors. This means CGM's international path is

unlikely to be scale-led and must instead rely on targeted country selection, ambulatory and pharmacy adjacency, and M&A or partnership-led entry.^{185 293}

- Investor read-through: Emerging markets should be viewed as medium-term option value rather than a near-term rerating catalyst. The more credible upside case is that CGM uses selective international expansion to add incremental growth and strategic scarcity value, while the core valuation still depends on defending and monetizing DACH and Western Europe.^{100 185}

8

Customer Satisfaction and Market Perception

8

Customer Satisfaction and Market Perception

Customer satisfaction is a material valuation variable in provider IT because replacement cycles are infrequent, implementations are disruptive, and referenceability strongly influences future bookings. Across the peer set, the 2025 to mid-2026 evidence shows a clear hierarchy: Epic and athenahealth continue to lead U.S. perception benchmarks, MEDITECH remains unusually resilient in its community and mid-market niche, Dedalus is the strongest independently visible hospital satisfaction benchmark in DACH and Southern Europe, and Oracle Health remains the clearest negative outlier on customer confidence. CGM sits in a more mixed position, with credible satisfaction signals in selected ambulatory revenue cycle workflows and a management narrative explicitly centered on rebuilding trust, but with materially less third-party benchmark visibility than the largest U.S. and European leaders.^{26 157 14 29}

8.1

KLAS Ratings and Independent Satisfaction Benchmarks

KLAS remains the most decision-relevant independent benchmark in this market because it captures direct user feedback and is widely used in board-level EHR selection processes. For mid-2026 positioning, the most important conclusion is that CGM is not absent from KLAS recognition, but its visibility is narrower and more workflow-specific than the suite-level leadership demonstrated by Epic, athenahealth, MEDITECH, and Dedalus.^{26 157}

Title — Selected 2026 satisfaction benchmarks

VENDOR	2026 BENCHMARK EVIDENCE	STRATEGIC READ THROUGH
Epic	Overall Best in KLAS for Health System Suites; EpicCare Ambulatory EHR scored 90.0 in health system owned ambulatory EHR.	Highest broad based satisfaction and strongest enterprise referenceability.
athenahealth	Overall Best in KLAS for Independent Physician Practice Suites; athenaClinicals scored 85.4 in independent ambulatory EHR for 11 to 75 physicians.	Premium ambulatory satisfaction benchmark with strong cloud and workflow credibility.
MEDITECH	Best in KLAS recognition for a 12th consecutive year; runner up for Overall Health System Suite.	Strong niche loyalty and durable customer partnership in community and mid market hospitals.
Oracle Health	2024 overall software suite ranking of 66.1 versus Epic 88.3 and MEDITECH 76.6; 2025 KLAS follow up found satisfaction had not materially improved.	Clear satisfaction laggard among scaled acute care vendors.
Dedalus	Ranked number 1 in 2026 Best in KLAS for Acute Care EHR in DACH and Southern Europe; DACH score reported at 73.5.	Strongest visible hospital satisfaction benchmark in CGM's core European theater.

VENDOR	2026 BENCHMARK EVIDENCE	STRATEGIC READ THROUGH
CGM	CompuGroup Medical US ARIA RCM scored 82.2 and won Best in KLAS in Ambulatory RCM Services associated with EHR.	Positive but narrower proof point, concentrated in ambulatory revenue cycle rather than broad suite leadership.

Source: ^{26 352 32 343}



■ Best in KLAS leader
 ■ RCM-focused score
 ■ Regional / niche score
 ■ Satisfaction laggard

Scores shown out of 100 per KLAS benchmarks. Oracle Health score from 2024 overall suite ranking. MEDITECH uses 2024 KLAS suite comparison score. Bars scaled to a 100-point axis.

- Epic: Epic's satisfaction lead is not just category specific. It won the 2026 Overall Best in KLAS award for Health System Suites, and its ambulatory score of 90.0 in the health system owned category reinforces that its reputation spans both acute and ambulatory enterprise workflows.^{26 157}
- athenahealth: athenahealth remains the strongest independent ambulatory perception benchmark. It won the 2026 Overall Best in KLAS award for Independent Physician Practice Suites and ranked first in five categories, indicating unusually broad satisfaction across physician office workflows rather than a single product module.^{25 157}
- MEDITECH: MEDITECH's 12th consecutive Best in KLAS recognition and runner up status for Overall Health System Suite matter disproportionately for investors because they signal that a subscale acute care vendor can still preserve premium customer sentiment if modernization is credible and execution is consistent.^{32 157}
- Oracle Health: Oracle remains the weakest large-vendor benchmark on customer perception. KLAS-based reporting showed a 2024 overall suite score of 66.1 versus Epic at 88.3 and MEDITECH at 76.6, and the 2025 Oracle Health follow-up concluded that customer satisfaction had not materially improved after the Cerner acquisition.^{34 38 115}
- Dedalus: In Europe, Dedalus is the most important independent satisfaction comparator for CGM in hospital IT. KLAS global awards for 2026 placed Dedalus ORBIS first in DACH and Dedalus Care first in Southern Europe, with Becker's citing a DACH score of 73.5, which supports the view that Dedalus currently has stronger hospital referenceability than CGM in its home region.^{349 352 343}

- CGM: The strongest independently visible satisfaction proof point located in this search is CGM US ARIA RCM, which scored 82.2 and won the 2026 Best in KLAS category for ambulatory RCM services associated with EHRs. That is a meaningful positive signal, but it does not yet establish CGM as a broad suite satisfaction leader in the way Epic, athenahealth, or Dedalus have done in their core categories.²⁶
- Evidence gap: No equally visible mid-2026 KLAS suite-level benchmark for CGM's core European ambulatory or hospital platforms was located in the sources reviewed. For investors, that lower benchmark visibility is itself relevant because it reduces third-party support for a premium market perception narrative relative to better-covered peers.^{26 14}

8.2

Switching Costs, Retention Rates, and Customer Loyalty Signals

In health IT, retention is usually inferred from migration behavior, recurring revenue durability, and customer willingness to stay on a vendor's go-forward platform rather than from consumer-style churn disclosures. On that basis, CGM's loyalty profile appears structurally solid in its installed base, especially given its 73 percent recurring revenue mix in 2025, but the strongest externally validated retention signals in the peer set still belong to Epic and MEDITECH, while Oracle Health shows the clearest erosion.^{14 29}

- CGM recurring revenue as loyalty proxy: CGM reported EUR 880.5 million of recurring revenue in 2025, equal to about 73 percent of sales. In enterprise software terms, that level of recurrence is a strong indicator of embedded workflows, contractual stickiness, and meaningful switching friction, even though the company does not publicly disclose a formal customer retention or net revenue retention metric in the sources reviewed.¹⁴
- CGM management signal: CGM's 2025 annual report states that the objective of its realignment is to increase customer satisfaction sustainably and establish CGM as the most trusted digital health platform in Europe and the United States. That wording implies management sees trust and satisfaction as active execution priorities rather than already-settled strengths, which is a more cautious signal than the market-leading posture of Epic or athenahealth.¹⁴
- Epic loyalty signal: KLAS reported that in 2025 only five acute care EHR vendors won validated net-new decisions, and both enterprise-wide decisions by health systems with more than 10 hospitals went to Epic. That pattern suggests not only strong new-logo momentum but also unusually high market trust in Epic as the lowest-risk long-duration platform choice.^{29 351}
- MEDITECH loyalty signal: KLAS highlighted that of 44 legacy MEDITECH customers making a go-forward decision in 2025, 84 percent chose to migrate to MEDITECH Expanse. For investors, that is one of the cleanest disclosed retention metrics in the sector and demonstrates that a legacy base can be defended if the migration path is credible.²⁹
- Oracle Health negative loyalty signal: Oracle Health has the clearest evidence of weakening customer loyalty. KLAS-based reporting said Oracle lost 57 acute care customers from 2022 to 2025, including 12 health systems with more than 1,000 beds, and about half of interviewed customers said they would not buy the EHR again.^{115 116}
- Dedalus loyalty signal: Dedalus' repeated number 1 KLAS ranking in DACH and Southern Europe for acute care EHR, now for a third consecutive year according to the company's 2026 release, is not a direct retention statistic but is a strong proxy for customer willingness to remain on and endorse the platform in hospital procurement cycles.^{343 349}

- Switching-cost conclusion for CGM: CGM's cross-setting footprint in ambulatory, pharmacy, and localized compliance workflows likely creates high practical switching costs, especially in Germany and adjacent Western European markets. However, because publicly available third-party retention metrics are sparse, the investment case rests more on inferred stickiness from recurring revenue and installed-base entrenchment than on the explicit migration-retention evidence available for MEDITECH or the broad satisfaction leadership visible for Epic.^{14 29}
- Disclosure caveat: No reliable mid-2026 public source located in this search disclosed a formal customer retention rate, gross revenue retention, or net revenue retention figure for CGM, Epic, Dedalus, eClinicalWorks, or NEXUS. That limits precision and means loyalty comparisons outside the KLAS migration evidence should be treated as directional rather than fully quantified.^{14 332 29}

8.3

Analyst and Investor Perception of CGM vs. Peers

Investor perception of health IT vendors is heavily shaped by a combination of customer satisfaction, installed-base durability, modernization credibility, and benchmark visibility. On that framework, CGM is generally perceived as a defensible but not premium-rated asset: stronger than distressed or trust-impaired platforms, but lacking the category leadership and perception flywheel that support superior strategic scarcity for Epic, athenahealth, or the best-positioned regional hospital leaders.^{14 157 29}

- CGM perception: CGM's public narrative is that of a trusted regional platform in transition. Management's stated goal to become the most trusted digital health platform in Europe and the United States, combined with heavy R&D investment and a high recurring revenue base, supports a turnaround-to-rerating thesis, but it also signals that trust expansion remains a work in progress rather than a completed strategic outcome.¹⁴
- Why benchmark visibility matters: Vendors that repeatedly win KLAS overall or category leadership awards benefit from a compounding perception advantage in both procurement and capital markets. Epic and athenahealth fit this pattern in 2026, while MEDITECH benefits from a long-running reputation for customer partnership that supports confidence despite smaller scale.^{157 32}
- Epic versus CGM: Epic is perceived as the premium strategic asset in provider IT because market share leadership and customer satisfaction reinforce each other. For investors, that combination implies lower churn risk, stronger pricing power, and superior referenceability in large-system tenders than CGM can currently claim.^{157 29}
- Oracle versus CGM: Oracle Health's weak customer sentiment has become an investor overhang rather than a temporary integration issue. KLAS-based reporting that satisfaction has stagnated, that 57 acute care customers were lost from 2022 to 2025, and that about half of interviewed customers would not buy again materially damages market confidence, which in relative terms makes CGM look more stable even if less globally scaled.^{115 116 351}
- Dedalus versus CGM: In European hospital IT, Dedalus currently appears to enjoy stronger external perception momentum because it combines regional scale with repeated KLAS recognition in DACH and Southern Europe. That matters for M&A and strategic investors because it suggests Dedalus has a more visible hospital leadership narrative, while CGM's differentiation remains broader but less sharply benchmarked.^{343 349}
- Practical investment read-through: CGM is likely to be viewed by investors as a quality-of-revenue story with upside from improved customer perception, not as a current best-in-class

satisfaction compounder. The rerating path therefore depends on converting recurring revenue and installed-base stickiness into more visible proof of customer advocacy, especially in core European ambulatory and hospital platforms where third-party benchmark evidence is still thinner than for peers.^{14 26}

- Bottom line: Relative to peers, CGM screens as investable and strategically relevant, but not yet perception-leading. In valuation terms, that usually supports a discount to premium satisfaction leaders and a premium to vendors with visible customer attrition, unless management can produce clearer evidence through 2026 and 2027 that trust, retention, and referenceability are improving at the platform level.^{26 115 14}

9

M&A Activity and Consolidation Landscape

9

M&A Activity and Consolidation Landscape

Health IT M&A through mid-2026 is being driven less by pure scale buying and more by platform densification around recurring workflow revenue, AI-enabled documentation, interoperability, and regional data-sovereignty positions. For CGM, the relevant question is not simply whether consolidation continues, but whether its combination of DACH density, 73 percent recurring revenue, and cross-setting assets makes it a disciplined bolt-on consolidator, a protected standalone compounder, or an increasingly scarce take-private or strategic target in a market where strategic buyers and sponsors remain active.^{370 365 17}

The broader consolidation pattern also remains bifurcated by geography and care setting. In U.S. acute care, concentration is already high and M&A is increasingly about adjacent workflow layers, while in Europe the market remains fragmented enough that regional champions such as Dedalus, NEXUS, CGM, Cambio, Softway Medical, and other country-specific vendors still have room to consolidate assets, win migrations, and build sovereign platforms around hospital and ambulatory modernization.^{369 363 364}

9.1

Recent M&A Transactions in Health IT and Their Strategic Rationale

Recent deal flow confirms that health IT buyers are prioritizing assets that deepen workflow control, add AI capability, or expand recurring revenue in adjacent provider operations rather than pursuing broad horizontal roll-ups alone. Levin Associates reported 63 eHealth M&A transactions in Q1 2026, up 21 percent from Q4 2025, with strategic investors accounting for 42 deals or 66 percent of activity, indicating that operating buyers remain the dominant force in sector consolidation.³⁷⁰

Title — Selected recent health IT consolidation signals

TRANSACTION OR SIGNAL	DATE	STRATEGIC RATIONALE
NEXUS acquisition of Omilon	March 2026	Adds AI-powered clinical documentation and speech technology to a European hospital software base, strengthening sovereign AI positioning.
BV Investment Partners investment in Moxe Health	March 2026	Backs an EHR-neutral clinical data exchange platform, reflecting continued value in interoperability infrastructure.
Dedalus 2025 contract momentum and 2026 market-share position	Reported July 2026	Demonstrates consolidation through hospital migrations and multiregional wins rather than only corporate acquisitions.
Q1 2026 eHealth M&A market	April 2026	Strategic buyers led activity, showing continued appetite for workflow, RCM, telehealth, and AI assets.

Source: ^{364 371 363 370}

- AI adjacency: NEXUS’s March 2026 acquisition of Omilon shows how European vendors are buying AI documentation capability rather than building every layer internally. NEXUS explicitly framed the deal around responsible AI, integration into established EHR environments, and the

need for stronger regional technology providers under European sovereignty and regulatory pressures.³⁶⁴

- Interoperability infrastructure: BV Investment Partners' majority investment in Moxe Health in March 2026 highlights continued sponsor appetite for EHR-neutral data exchange and payer-provider workflow assets. That is strategically relevant for CGM because interoperability layers can become high-multiple control points even when they do not own the core EHR.³⁷¹
- Hospital platform densification: Dedalus's July 2026 disclosure that it recorded the highest number of contracted hospital beds for the fourth consecutive year, with almost 10,000 in 2025 and 1,637 inpatient hospitals served across multiple regions, shows that consolidation is also occurring through migrations and contract wins that increase installed-base gravity without requiring a new headline acquisition each time.³⁶³
- Capital backdrop: Bain said global healthcare private equity reached a record roughly USD 191 billion of deal value in 2025, with provider and biopharma segments leading and activity driven in part by healthcare IT. That backdrop supports continued competition for scaled software assets with recurring revenue and clear value-creation levers.³⁶⁵
- Strategic implication for CGM: The most relevant read-through is that buyers are paying for control of workflow, data exchange, AI-enabled productivity, and regional compliance moats. CGM participates in all four themes, which increases its strategic relevance even if it is not the largest consolidator in the field.^{17 364}

9.2

CGM's Own Acquisition History and Integration Track Record

CGM's acquisition history is best characterized as steady portfolio assembly rather than transformational megadeals. The company has repeatedly used small and mid-sized acquisitions and asset deals to add customer bases, local service capacity, workflow modules, and geographic density, especially in ambulatory and adjacent provider software.¹⁷

The 2024 annual report shows that CGM remained active even during a period of operational realignment. New 2024 combinations included AmbulApps GmbH, Pridok AS, CPS Concept SAS, MARITEC, team2work GmbH, GeMaMed GmbH, Manhattan Billing and Collections, Medical Services Associates, and CGM Development HUB GmbH, while the report also references prior acquisitions such as m.Doc, Meta-it, Schuyler House, Pre-Billing Consultants, and Innomed through contingent payments and purchase price allocation updates.¹⁷

- Strategic pattern: The disclosed rationale across many deals is highly consistent, namely extension of the customer platform, expansion of market reach, improvement of professional services, and establishment of stronger sales and service structures. That consistency suggests disciplined bolt-on logic rather than opportunistic diversification.¹⁷
- 2024 ambulatory emphasis: Several 2024 additions were clearly AIS-oriented, including MARITEC, Manhattan Billing and Collections, and Medical Services Associates in the United States, plus GeMaMed and team2work in German-speaking markets. This indicates that CGM still sees ambulatory and revenue-cycle-adjacent workflows as the most efficient area for tuck-in M&A.¹⁷
- Capability expansion: Pridok and AmbulApps are more strategically important than their size alone suggests because they support CGM's cloud, patient communication, and digital front-door agenda. The completed 2024 purchase price allocation for m.Doc likewise points to CGM's

continued effort to strengthen patient engagement and digital workflow capabilities around the installed base.¹⁷

- Integration quality: Public disclosure does not provide a clean acquisition-by-acquisition ROI scorecard, so any judgment must be inferential. The evidence supports a mixed but broadly competent integration record, with CGM able to absorb numerous small assets into recurring revenue streams, but still carrying the portfolio complexity and modernization burden that come from years of cumulative bolt-ons.^{17 100}
- Evidence of continued bolt-on appetite: CGM's March 5, 2026 press release on acquiring medical publisher perimed indicates that management remains willing to pursue small strategic additions even while focusing on product innovation and operational execution. The target is not core EHR software, but it fits the pattern of adding adjacent content and workflow assets around existing customer relationships.³⁶⁸
- Investor assessment: CGM has historically been more effective at buying and retaining niche customer bases than at rapidly simplifying the resulting architecture. For M&A professionals, that means CGM's track record supports further tuck-ins in ambulatory, pharmacy, AI documentation, and country-specific compliance software, but argues against a large, highly leveraged transformational acquisition in the near term.^{17 100}

9.3

Potential Consolidation Scenarios and CGM's Role as Acquirer or Target

The most plausible consolidation scenarios for CGM over the next several years fall into three buckets: continued bolt-on acquisitions in Europe, selective portfolio reshaping under private ownership, or a larger strategic transaction if a buyer seeks immediate scale in DACH and Western European provider IT. Which path dominates will depend on whether CGM can convert its installed base into faster cloud and AI monetization before larger rivals or sponsors move more aggressively.^{100 365}

- Scenario one, disciplined acquirer: CGM remains the most natural buyer of subscale ambulatory, pharmacy, RCM, and compliance software assets in DACH and adjacent European markets. Its recurring revenue base, local distribution, and history of integrating small assets make this the lowest-risk path, especially where targets can be migrated onto CGM ONE, pharmacy cloud products, or shared service infrastructure.^{17 100}
- Scenario two, regional platform consolidation: Europe remains fragmented enough that larger regional combinations are strategically conceivable. KLAS's 2026 global hospital market-share summary shows large European decisions going to vendors including NEXUS AG, Softway Medical, Numih France, Cambio, and Dedalus, which implies a still-diverse field where scale, interoperability investment, and sovereign AI capability could drive further combinations.³⁶⁹
- Scenario three, CGM as target for financial sponsors: Sponsor appetite remains credible given record healthcare private equity deal value in 2025 and the sector's preference for recurring software assets with operational improvement levers. CGM's installed base, recurring revenue profile, and margin-improvement opportunity fit a classic sponsor thesis, particularly if private ownership is viewed as a better environment for portfolio simplification and longer-duration product migration.³⁶⁵
- Scenario four, CGM as target for a strategic buyer: A larger strategic acquirer could view CGM as a scarce route into German ambulatory, pharmacy, and selected hospital workflows. The strategic logic would be strongest for a buyer lacking DACH density but seeking recurring

revenue, regulatory localization, and cross-setting customer access, though antitrust, integration complexity, and product overlap would be material diligence issues.^{100 363}

- Why CGM is not the most likely large-scale predator: Compared with Dedalus, which now says it serves 1,637 inpatient hospitals globally, or NEXUS, which is already buying AI documentation capability, CGM appears better positioned for targeted tuck-ins than for leading a pan-European hospital roll-up from here. Its balance between recurring revenue quality and ongoing modernization needs argues for selectivity rather than aggressive balance-sheet expansion.^{363 364 100}
- Bottom line for investors: CGM's highest-probability role is a selective consolidator in fragmented European ambulatory and pharmacy software, while its highest-upside optionality is as a scarce strategic or sponsor target if buyers decide that DACH workflow density and recurring revenue are worth paying for. The key determinant of bargaining power will be execution, because stronger cloud migration, AI commercialization, and margin recovery would make CGM more credible both as an acquirer and as a premium target.^{100 370 365}

10

SWOT Analysis: CGM vs. the Competitive Field

10

SWOT Analysis: CGM vs. the Competitive Field

This SWOT assessment frames CompuGroup Medical as a defensible but execution-sensitive European health IT platform. Relative to global leaders and regional peers, CGM's investment case rests on recurring revenue quality, DACH and Western Europe workflow entrenchment, and cross-setting breadth across ambulatory, hospital, and pharmacy software, while the principal debate is whether management can convert those assets into faster cloud migration, stronger customer advocacy, and higher-margin platform economics by 2026 and beyond.¹⁰⁰

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For investors and M&A professionals, the key conclusion is that CGM is stronger as a regional installed-base compounder than as a global category leader. Its upside lies in monetizing local density, regulatory fit, and AI-enabled workflow automation before larger suite vendors, hospital-centric European consolidators, or cloud-native ambulatory platforms compress its strategic room for maneuver.^{100 343 250}

10.1

CGM's Core Strengths and Defensible Competitive Advantages

CGM's strongest competitive asset is the quality and breadth of its installed base. In FY2025, the company generated EUR 1.213 billion of revenue, of which EUR 880.5 million was recurring, or about 73 percent of sales, across ambulatory, hospital, and pharmacy workflows, giving it a more diversified care-setting footprint than most regional peers and more revenue visibility than many private competitors publicly disclose.^{100 14}

- Revenue quality: CGM's recurring mix of about 73 percent in 2025 remains a core valuation support because it compares favorably with NEXUS's roughly 53.8 percent maintenance-led revenue mix in 2024 and provides stronger visibility than is publicly available for private peers such as Dedalus, athenahealth, and eClinicalWorks.^{100 216}
- Geographic defensibility: Germany contributed EUR 684.5 million and Western Europe EUR 356.1 million of FY2025 revenue, meaning about 86 percent of group sales still came from CGM's home theater, where local reimbursement logic, compliance requirements, and workflow customization raise switching friction.¹⁰⁰
- Cross-setting breadth: Unlike hospital-pure-play rivals or ambulatory-only vendors, CGM can cross-sell across physician offices, hospitals, pharmacies, and adjacent workflow services. That breadth is strategically valuable in Europe, where procurement remains fragmented and country-specific integration still matters.^{100 250}
- Pharmacy economics: PCS generated EUR 156.0 million of 2025 revenue at a 32 percent adjusted EBITDA margin, making pharmacy one of CGM's most attractive verticals from a cash generation and retention standpoint relative to lower-margin hospital software.¹⁰⁰
- Regulatory and infrastructure fit: CGM's embedded role in German e-prescription and practice connectivity workflows creates a practical moat that global vendors cannot easily replicate through generic product localization alone.¹⁰⁰
- Innovation capacity with installed-base leverage: CGM spent EUR 274 million on R&D in 2025, or roughly 22.6 percent of revenue, which is high for a mature health IT vendor and gives it the

capacity to modernize core products while monetizing AI and cloud add-ons into an already captive customer base.¹⁰⁰

- Narrow but positive third-party proof points: CGM's U.S. ARIA RCM Services won 2026 Best in KLAS in ambulatory RCM services associated with EHRs with an 82.2 score, showing that the company can still produce category-leading customer outcomes in selected workflow niches.^{386 387}

Title — CGM strategic strengths snapshot

STRENGTH AREA	EVIDENCE	INVESTOR IMPLICATION
Recurring revenue quality	EUR 880.5m recurring revenue, about 73% of FY2025 sales.	Supports cash flow visibility and valuation resilience.
Home-market density	Germany plus Western Europe represented about 86% of FY2025 revenue.	Reinforces local moat and lowers competitive displacement risk in core markets.
Cross-setting portfolio	Exposure across AIS, HIS, and PCS.	Creates cross-sell optionality and diversified workflow control points.
High-value pharmacy vertical	PCS margin reached 32% in 2025.	Provides a premium earnings pool within the portfolio.
R&D commitment	EUR 274m R&D spend in 2025, about 22.6% of revenue.	Increases probability of successful cloud and AI migration if execution holds.

Source: ¹⁰⁰

10.2

Key Vulnerabilities and Competitive Gaps Relative to Global Peers

CGM's main weakness is not lack of assets but uneven strategic coherence relative to best-in-class peers. Compared with Epic's integrated enterprise suite, athenahealth's cloud-native ambulatory operating model, or Dedalus's hospital-centric density in DACH, CGM still carries a heavier burden of portfolio complexity, modernization overlap, and mixed customer proof points.^{100 343}

- Margin gap: CGM's FY2025 adjusted EBITDA margin was 16 percent, below NEXUS's 2024 EBITDA margin of about 22.0 percent and likely below what investors would infer for highly scaled private leaders such as Epic. This suggests that CGM's broad portfolio still carries transformation and service-intensity drag.^{100 216}
- Hospital competitiveness: Dedalus remains the stronger visible hospital benchmark in DACH, with repeated Best in KLAS recognition for acute care EHR in the region. That weakens CGM's claim to hospital leadership in its own home market.³⁴³
- Limited suite-level benchmark visibility: The strongest public KLAS proof point for CGM is ARIA RCM in the U.S., not a broad European ambulatory or hospital suite award. By contrast, Epic, athenahealth, MEDITECH, and Dedalus all have more visible category or suite-level recognition.^{386 343}
- Cloud transition burden: CGM is progressing through CGM ONE, CGM Praxis, and cloud pharmacy initiatives, but it is still migrating a heterogeneous installed base rather than operating

from a clean cloud-native architecture. Recent technical literature on FHIR transformation and cloud-native clinical workloads underscores how much execution discipline is required to industrialize interoperability and analytics at scale.^{100 250 251}

- U.S. scale disadvantage: North America contributed only EUR 133.4 million of FY2025 revenue, leaving CGM structurally subscale against Epic, Oracle Health, athenahealth, eClinicalWorks, and MEDITECH in the world's most reference-driven provider IT market.¹⁰⁰
- Balance-sheet constraint: CGM entered this transformation period with elevated leverage in prior disclosures, which limits tolerance for execution slippage and reduces flexibility for large-scale strategic moves relative to better-capitalized global rivals.^{17 100}
- Strategic narrative gap: Investors can understand Epic as the premium enterprise suite, athenahealth as the cloud ambulatory leader, and Dedalus as the European hospital consolidator. CGM's broader but less unified story can dilute procurement clarity and valuation premium unless management demonstrates faster platform convergence.^{100 250}

10.3

Opportunities in Underserved Markets, Segments, and Geographies

CGM's best opportunities are concentrated where fragmentation, regulation, and workflow localization still matter more than global brand power. That points to European ambulatory, pharmacy, and mid-market hospital modernization, plus selective international expansion in markets where cloud migration and interoperability mandates are opening replacement cycles without yet being fully captured by Epic-style enterprise platforms.^{100 250}

- DACH ambulatory upsell: CGM can still deepen monetization in its physician base through CGM Praxis, TelefonAssistent, Self CheckIn, DokuAssistent, and related CGM ONE modules. This is likely the highest-return growth vector because it leverages existing customer relationships rather than expensive new-logo acquisition.¹⁰⁰
- Pharmacy cloud migration: PCS combines high retention with superior margins, and cloud products such as CGM STELLA create a path to lower support intensity and higher ARPU. Relative to hospital core systems, pharmacy remains a more attractive capital-light expansion lane.¹⁰⁰
- SAP IS-H successor demand and hospital RCM: CGM CLINICAL RCM and the broader G3-based hospital stack position the company to capture administrative and revenue-cycle modernization budgets in German-speaking hospitals facing legacy transition pressure. This is one of the clearest near-term hospital openings available to regional incumbents.¹⁰⁰
- Selective Eastern Europe and adjacent markets: CGM already generated EUR 33.5 million of FY2025 revenue in Eastern Europe, providing a foothold for expansion into less consolidated provider IT markets where local adaptation and total cost of ownership remain decisive.¹⁰⁰
- AI-enabled installed-base monetization: CGM does not need to outspend Epic or Oracle to create value. It can use ambient documentation, summarization, call automation, and workflow copilots to raise retention and wallet share inside existing accounts, especially in ambulatory and pharmacy settings.^{100 251}
- Interoperability as a monetizable layer: Technical progress in reusable FHIR and openEHR transformation frameworks suggests that vendors able to standardize mappings and reduce custom integration work can improve both product stickiness and implementation economics. CGM's cross-setting footprint gives it a credible base to productize this capability in Europe.²⁵⁰

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- M&A whitespace: Fragmented European ambulatory, pharmacy, and compliance software markets still offer tuck-in targets that can be migrated onto CGM's platform. This remains a more realistic value-creation path than large-scale transformational M&A.^{17 100}

10.4

Threats from Global Platform Players, New Entrants, and Disruptive Models

The external threat environment is intensifying because the market is consolidating around fewer strategic platforms while AI lowers the perceived differentiation of legacy front-end workflows. CGM is therefore exposed both to large incumbents with superior capital and referenceability, and to modular challengers that can attack high-value workflow layers without replacing the full core system.^{343 250}

- Epic and premium enterprise encroachment: Epic remains the benchmark for large-system acute care execution and customer trust. Even where it is not a direct ambulatory or pharmacy competitor, its presence in flagship hospital tenders can reset buyer expectations around integration, AI, and implementation quality.³⁴³
- Dedalus as the most immediate European threat: Dedalus's repeated number one KLAS position in DACH acute care EHR and its hospital-centric scale make it the clearest structural threat to CGM's hospital ambitions in Western Europe. It is the competitor most likely to compress CGM's pricing power and referenceability in regional hospital procurements.³⁴³
- Cloud-native ambulatory challengers: athenahealth and eClinicalWorks demonstrate how integrated cloud EHR, PM, RCM, and patient engagement stacks can outcompete more heterogeneous architectures on speed, usability, and release cadence. Their model raises the risk that CGM's ambulatory base could face margin pressure if cloud migration is too slow.^{386 100}
- Oracle optionality risk: Oracle Health is currently weaker on customer sentiment than Epic, but Oracle's capital base and OCI-led rebuild mean it cannot be dismissed. If the platform reset succeeds after 2026, Oracle could re-enter European and international hospital cycles with aggressive bundling and AI-led positioning.¹⁰⁰
- Interoperability-driven modular disruption: As FHIR tooling, transformation engines, and API ecosystems mature, customers may buy best-of-breed workflow layers without committing to a single incumbent suite. That can erode the historical moat of installed-base vendors if they do not own the most valuable automation and data-exchange layers.^{250 253}
- AI abstraction risk: Ambient documentation, summarization, and patient communication are increasingly being delivered by specialized vendors and infrastructure partners. If these layers become vendor-agnostic, some of CGM's planned AI differentiation could be competed away into lower-margin feature parity.²⁵¹
- Regulatory cost inflation: European and U.S. interoperability, security, and AI governance requirements raise barriers to entry, but they also increase compliance cost for incumbents with broad legacy estates. Larger global players may absorb those costs more efficiently than CGM can.^{5 100}

The investor takeaway is that CGM's threat profile is less about sudden displacement and more about gradual strategic compression. If management fails to accelerate platform unification, cloud conversion, and customer proof points, the company risks remaining a durable but lower-multiple regional incumbent while better-capitalized or more coherent rivals capture the premium layers of future value creation.^{100 216 343}

Valuation and Investment Considerations

11

Valuation and Investment Considerations

CGM's valuation case in mid-2026 is best understood as a quality-of-revenue asset trading against a mixed execution profile. Relative to listed software and health IT comparables, the company benefits from a high recurring revenue base and defensible DACH positioning, but its multiple support is constrained by lower margins than best-in-class software peers, a still-elevated transformation burden, and limited public benchmark visibility versus premium assets such as Epic or athenahealth, which remain private and therefore unavailable for direct public-market multiple comparison.^{100 431 211}

For investors and M&A professionals, the central issue is whether CGM should be underwritten as a steady regional compounder, a margin-recovery story, or a strategic scarcity asset. The answer depends on three variables: the pace of cloud and AI monetization inside the installed base, the degree to which hospital and ambulatory modernization can lift EBITDA conversion, and whether strategic buyers or sponsors would pay a control premium for CGM's cross-setting footprint in Germany and Western Europe.^{100 370 365}

11.1

CGM Valuation Multiples vs. Listed Health IT Peers

Public-market evidence supports a discount-to-premium-software but premium-to-distressed-legacy framing for CGM. MarketScreener shows CGM at roughly EUR 1.42 billion market capitalization in mid-2026, while the 2025 annual report shows EUR 720.1 million of net debt, EUR 1.213 billion of revenue, and EUR 240 million of adjusted EBITDA for FY2025, implying an enterprise value of about EUR 2.14 billion, EV/Revenue of about 1.8x, and EV/EBITDA of about 8.9x on FY2025 figures.^{431 100 21}

That valuation sits below what public markets typically award to higher-growth, cleaner-margin software platforms, but it is not distressed relative to European healthcare IT assets with slower growth and heavier services exposure. The multiple is broadly consistent with a company that has strong recurring revenue quality at about 73 percent of sales, but only a 16 percent adjusted EBITDA margin and a portfolio still absorbing modernization costs.¹⁰⁰

Title — Indicative listed peer valuation comparison

COMPANY	EV/REVENUE	EV/EBITDA	READ THROUGH
CGM	~1.8x	~8.9x	Recurring revenue support offset by lower margin and transformation risk.
NEXUS AG	~6.0x	~27.1x	Premium reflects stronger margin profile, narrower scope, and scarcity value before/through take-private context.
Oracle	~9.0x to 9.8x	Not meaningful for Oracle Health standalone	Parent multiple reflects hyperscale cloud and applications mix, not a pure health IT comp.

COMPANY	EV/REVENUE	EV/EBITDA	READ THROUGH
Dedalus	Not publicly disclosed	Not publicly disclosed	Private sponsor-backed asset, so direct public multiple comparison is unavailable.

Source: 431 100 421 211 404 235

- Relative to NEXUS: NEXUS reported EUR 261.5 million of 2024 revenue and EUR 57.6 million of EBITDA, while CompaniesMarketCap shows about USD 1.47 billion of market capitalization in August 2026 and notes the company was acquired by TA Associates. Even allowing for currency and net debt approximation limits, NEXUS screens at a materially higher revenue and EBITDA multiple than CGM, indicating that public and transaction markets have rewarded its cleaner profitability and focused hospital software profile more aggressively than CGM's broader but more complex platform.^{211 421}
- Relative to Oracle: Oracle's enterprise value was reported around USD 514 billion to USD 564 billion in August 2026, and its FY2026 revenue was USD 57.4 billion. That implies an EV/Revenue range of roughly 9.0x to 9.8x, but Oracle is only a partial comp because the multiple reflects cloud infrastructure, database, and enterprise applications rather than Oracle Health alone.^{404 407 235}
- Relative to private peers: Epic, athenahealth, eClinicalWorks, MEDITECH, and Dedalus do not provide sufficiently current public valuation disclosures for a reliable like-for-like multiple table in this search. For investor purposes, that means CGM's most defensible direct listed comparison remains NEXUS, while Oracle is useful only as an upper-bound strategic platform reference rather than a pure health IT valuation anchor.^{239 146 135}
- Multiple implication: CGM's current trading frame suggests the market is capitalizing its installed-base durability, but not yet paying fully for a successful cloud and AI transition. A rerating would likely require evidence that recurring revenue can remain above 70 percent while EBITDA margin recovers toward the low-20s rather than remaining in the mid-teens.¹⁰⁰

11.2

Key Financial Risks and Return Drivers for Investors and M&A Professionals

The main financial risk is not demand collapse but execution slippage against a valuation that already assumes some normalization. CGM's 2025 annual report shows revenue of EUR 1.213 billion, adjusted EBITDA of EUR 240 million, net income of EUR 29 million, and net debt of EUR 720.1 million, which leaves limited room for prolonged margin underperformance if R&D intensity, implementation costs, or restructuring drag remain elevated.^{100 21}

- Margin recovery risk: Group adjusted EBITDA margin was 16 percent in 2025, below the 23 percent AIS margin and 32 percent PCS margin because hospital and transformation activities dilute profitability. If HIS modernization remains service-heavy or delayed, the group may struggle to convert revenue growth into valuation-accretive EBITDA expansion.¹⁰⁰
- Leverage and capital allocation risk: Net debt fell from EUR 772.8 million at year-end 2024 to EUR 720.1 million in 2025, but leverage still constrains strategic flexibility relative to larger peers and reduces tolerance for a large acquisition misstep or a prolonged downturn in hospital project conversion.^{17 100}
- Portfolio complexity risk: CGM's multi-setting footprint is strategically valuable, but it also requires simultaneous investment across ambulatory, hospital, pharmacy, cloud, AI, and compliance layers. The company spent EUR 274 million on R&D in 2025, about 22.6 percent of

revenue, which is supportive for long-term competitiveness but suppresses near-term earnings conversion if monetization lags.¹⁰⁰

- Regulatory and compliance cost risk: U.S. HTI rules, the EHDS framework, and the EU AI Act all raise the cost of maintaining compliant product roadmaps. These rules can strengthen incumbent moats, but they also favor vendors with greater scale and cleaner architectures if compliance work becomes a recurring margin headwind.^{74 69 71}
- Customer proof-point risk: CGM has positive niche validation, including ARIA RCM's 2026 Best in KLAS recognition, but it lacks the broad suite-level third-party validation that supports premium pricing and lower perceived churn risk for Epic, athenahealth, or Dedalus in their core categories.^{26 343}

The principal return drivers are more attractive than the risk list suggests, but they are concentrated in a few specific levers rather than broad-based market share gains.

- Installed-base monetization: Recurring revenue of EUR 880.5 million, or about 73 percent of 2025 sales, remains the core valuation support. If CGM can layer AI documentation, patient communication, cloud hosting, and workflow automation onto this base, revenue quality can improve without requiring aggressive new-logo acquisition.¹⁰⁰
- Mix shift toward higher-value verticals: PCS generated EUR 156.0 million of revenue at a 32 percent adjusted EBITDA margin in 2025, making pharmacy one of the clearest sources of incremental value creation. A greater share of growth from pharmacy and ambulatory cloud modules would likely be multiple-accretive.¹⁰⁰
- Hospital modernization optionality: HIS revenue grew 10 percent organically in 2025 to EUR 246.8 million, supported by SAP IS-H successor demand and CGM CLINICAL RCM positioning. If that growth can be standardized and delivered with less services intensity, the segment could move from strategic drag to rerating catalyst.^{100 92}
- M&A and control premium support: Strategic buyers and sponsors remain active in health IT, with 63 eHealth M&A deals in Q1 2026 and strategic buyers representing 66 percent of activity. That backdrop supports downside valuation protection for scaled recurring-revenue assets, especially those with regional scarcity in Europe.^{370 365}
- M&A diligence implication: For a buyer, the underwriting focus should be on gross retention durability, migration economics, and the cost to rationalize overlapping products. The strategic value is real, but so is the risk that integration and platform simplification consume more capital and time than a headline multiple alone implies.^{100 359}

11.3

Strategic Scenarios: Standalone Growth, Partnership, or Acquisition

The most credible strategic scenarios for CGM are not symmetrical. Standalone execution remains the base case, partnership is the most capital-efficient accelerator, and a full acquisition is the highest-upside but lower-probability path unless valuation remains modest relative to strategic scarcity.

- Standalone growth case: This is the highest-probability scenario. Under this path, CGM continues to prioritize low- to mid-single-digit organic growth in 2026, modest cash EBITDA margin improvement, and installed-base monetization through CGM ONE, CGM Praxis, pharmacy cloud migration, and hospital RCM modernization.^{100 92}

- Standalone valuation outcome: If management can sustain revenue growth around guidance, keep recurring revenue above 70 percent, and lift group EBITDA margin from 16 percent toward the high teens or low 20s, CGM could plausibly narrow part of the valuation gap to focused European software peers. The rerating would come more from improved earnings quality than from aggressive top-line acceleration.^{100 211}
- Partnership case: Selective partnerships in AI, interoperability, and cloud infrastructure may offer the best risk-adjusted route to faster product modernization. This is consistent with broader sector behavior, where vendors increasingly partner or buy for AI documentation, data exchange, and workflow automation rather than building every layer internally.^{364 371}
- Partnership valuation outcome: A partnership-led model could improve CGM's product velocity without materially increasing leverage, which would be positive for both public investors and strategic buyers. The trade-off is that some economics would be shared, so the market would need evidence that partnerships improve retention, ARPU, or implementation efficiency rather than simply adding features.¹⁰⁰
- Acquisition or take-private case: CGM remains a plausible target for either private equity or a larger strategic buyer because it offers scale in German ambulatory and pharmacy software, meaningful recurring revenue, and cross-setting access points that are difficult to replicate organically. Record healthcare PE deal value in 2025 and continued strategic-buyer activity in 2026 support the feasibility of this scenario.^{365 370}
- Acquisition valuation outcome: A buyer could justify paying above CGM's current public-market trading level if it underwrites synergy from platform simplification, procurement leverage, cloud migration, and cross-sell into the installed base. However, the control premium would be capped by integration complexity, antitrust considerations in DACH niches, and the need to preserve customer trust during any ownership transition.^{100 359}
- Bottom line: For public investors, the most defensible underwriting case is a standalone margin-recovery and installed-base monetization story with strategic optionality. For M&A professionals, CGM is more attractive as a scarce regional platform asset than as a pure growth compounder, which means transaction value will depend less on headline revenue growth and more on confidence in migration economics, retention durability, and post-deal simplification potential.^{100 370}

Strategic Outlook and Recommendations

12

Strategic Outlook and Recommendations

CGM's post-2026 strategic outlook is best framed as a contest between installed-base monetization and strategic compression. The company retains meaningful advantages in DACH and Western Europe through recurring revenue, ambulatory and pharmacy density, and localized workflow fit, but its ability to convert those assets into a higher-quality growth and valuation profile will depend on faster platform convergence, clearer customer proof points, and more disciplined capital allocation than many peers have required.^{100 29 344}

For investors and M&A professionals, the central conclusion is that CGM should not be underwritten as a broad global share gainer. It is more credibly positioned as a regional platform that can create value by defending core markets, selectively taking share in specific modernization windows, and improving revenue quality and margin conversion through cloud, AI, and interoperability-led upsell.^{444 250 251}

12.1

CGM's Competitive Trajectory for 2026 and Beyond

CGM's likely trajectory beyond 2026 is that of a more focused, higher-recurring European healthcare software platform, but not a category leader across all care settings. The company's FY2025 revenue of EUR 1.213 billion, recurring revenue of EUR 880.5 million, and management guidance for low to mid single-digit organic growth with slight cash EBITDA margin improvement indicate a stabilization and rebuilding phase rather than a breakout share-capture cycle.^{100 444}

- Base-case trajectory: CGM remains strongest as a DACH and Western Europe incumbent with improving cloud and AI packaging, but only selective new-logo momentum outside its installed base. This follows from its revenue concentration in Germany and Western Europe, where local workflow depth still matters more than global brand scale.¹⁰⁰
- Competitive pressure in hospitals: The hospital segment will remain the most contested part of the portfolio. Epic continues to dominate U.S. acute care with 43.7 percent hospital share in 2025, while Dedalus retained the number one KLAS position in DACH acute care EHR for the third consecutive year, limiting CGM's probability of becoming the reference hospital platform across large systems.^{117 344}
- Ambulatory and pharmacy trajectory: These remain the most credible engines of value creation. CGM's AIS generated EUR 810.4 million of 2025 revenue and PCS generated EUR 156.0 million at a 32 percent adjusted EBITDA margin, which supports a thesis of continued wallet-share expansion through cloud migration and workflow automation rather than disruptive market expansion.¹⁰⁰
- Technology trajectory: CGM is directionally aligned with where the market is moving, especially around FHIR-based interoperability, cloud-native delivery, and AI-enabled workflow compression. Recent arXiv work on standardized openEHR to FHIR transformation and event-driven cloud-native clinical architectures reinforces that future competitiveness will depend on reusable mappings, lower custom integration burden, and scalable data pipelines, all areas where CGM has a credible but still incomplete modernization agenda.^{250 253 251}
- Strategic implication: Post-2026, CGM can plausibly improve its strategic standing without becoming a global winner-take-all platform. The most realistic upside is a rerating from complex

regional incumbent to more coherent European platform asset, provided management can show that R&D intensity is translating into faster migrations, better customer advocacy, and higher segment margins.^{100 211}

12.2

Where CGM Can Realistically Win, Hold, or Lose Ground

Title — CGM competitive battlegrounds after 2026

AREA	LIKELY OUTCOME	STRATEGIC RATIONALE
DACH ambulatory software and add-on workflows	Win	Installed-base density, regulatory localization, and cloud plus AI upsell create the clearest path to ARPU expansion.
Pharmacy software in Europe	Win	High retention and 32 percent PCS adjusted EBITDA margin make pharmacy the most attractive niche for margin-accretive growth.
Mid-market hospital RCM and SAP IS-H successor workflows in German-speaking markets	Hold to selective win	CGM has a time-sensitive opening, but execution and referenceability must improve to convert it.
Large flagship hospital core EHR tenders	Lose	Epic and Dedalus remain stronger reference vendors, with Oracle still relevant despite weaker sentiment.
U.S. broad ambulatory and acute care share	Lose	Market concentration and KLAS leadership by Epic and athenahealth make broad-based expansion unrealistic.
European interoperability and workflow middleware layers	Hold to selective win	Open standards and FHIR tooling create room for modular monetization if CGM productizes integration rather than custom services.

Source: ^{100 117 29 344 250}

- Win: CGM can most realistically gain ground in German and adjacent European ambulatory workflows where cloud migration, patient communication, documentation automation, and compliance tooling can be sold into an existing customer base. This is the highest-return path because it relies on installed-base monetization rather than expensive displacement selling.¹⁰⁰
- Win: Pharmacy remains the cleanest earnings pool in the portfolio. With PCS at EUR 156.0 million revenue and 32 percent adjusted EBITDA margin in 2025, CGM has room to deepen cloud conversion and adjacent data services in a vertical where global mega-vendors are less structurally dominant.¹⁰⁰
- Hold: CGM can defend meaningful share in mid-market hospitals and administrative modernization programs, especially around CGM CLINICAL RCM and SAP IS-H successor demand. However, this is more a defend-and-upgrade opportunity than a likely route to outright hospital leadership.¹⁰⁰
- Hold: Interoperability and workflow middleware are realistic areas of selective advantage if CGM standardizes them into products. Recent technical literature suggests that reusable FHIR

mappings and automated transformation tools can reduce custom ETL dependence, which could let CGM defend accounts even where it is not the dominant front-end EHR.^{250 253}

- Lose: CGM is likely to lose relative ground in large enterprise hospital core-system competitions. Epic's 43.7 percent U.S. acute care hospital share and Dedalus's repeated KLAS leadership in DACH acute care EHR indicate that the premium hospital reference market is already concentrated around stronger brands and clearer platform narratives.^{117 344}
- Lose: In the United States, CGM should assume continued subscale positioning. KLAS shows acute care decisions concentrated among a handful of vendors, and federal rulemaking commentary indicates Epic, Oracle Cerner, and athenahealth together account for about 82 percent of clinician market share, leaving little room for a broad CGM expansion thesis.^{29 192}

12.3

Strategic Recommendations for Positioning, Differentiation, and Value Creation

CGM's strategic agenda should prioritize fewer, more monetizable control points rather than broad portfolio ambition. The company already has enough installed-base breadth to matter strategically, but the next phase of value creation depends on simplifying the story for customers, investors, and potential acquirers.^{100 364}

- Prioritize ambulatory and pharmacy as the primary value-creation core: Management should explicitly allocate capital and go-to-market focus toward AIS and PCS, where recurring revenue, retention, and margin structure are strongest. For investors, this would improve mix quality and reduce dependence on lower-margin hospital project work.¹⁰⁰
- Reframe hospital strategy around selective control points, not full-spectrum leadership: CGM should compete where it has a timing or localization edge, especially hospital RCM, administrative modernization, and SAP IS-H successor pathways, while avoiding capital-intensive attempts to match Epic or Dedalus across every acute-care workflow. This would improve win rates and reduce implementation risk.^{100 344}
- Productize interoperability as a margin lever: CGM should turn integration capability into standardized middleware, mapping libraries, and deployment accelerators rather than continuing to absorb it as bespoke services work. The technical direction supported by recent arXiv literature suggests that reusable transformation frameworks can lower delivery cost, improve stickiness, and create a differentiated European data-liquidity layer.^{250 253}
- Use AI to deepen retention and ARPU, not to chase headline parity: CGM should focus AI investment on ambient documentation, call automation, summarization, coding support, and patient communication inside existing workflows. This is more realistic than trying to outbuild Oracle or out-reference Epic, and it aligns with the broader evidence that scalable healthcare AI depends on workflow integration and data governance rather than model novelty alone.^{251 100}
- Tighten M&A criteria: Future acquisitions should be limited to tuck-ins that accelerate cloud migration, AI documentation, pharmacy adjacency, or country-specific compliance depth. CGM's own history and the broader market, including NEXUS's Omilon acquisition, suggest that capability densification is more value-accretive than large-scale horizontal expansion at this stage.^{364 100}
- Create explicit investor milestones for rerating: Management should report progress against a small set of externally legible KPIs, including cloud migration penetration, attach rates for AI and workflow modules, recurring revenue durability above 70 percent, and margin improvement by

segment. Without clearer proof points, the market is likely to continue valuing CGM as a durable but lower-multiple incumbent rather than a strategic scarcity asset.¹⁰⁰

- Strategic end-state recommendation: CGM should position itself as the leading sovereign workflow and interoperability platform for ambulatory, pharmacy, and selected hospital-adjacent processes in DACH and Western Europe. That positioning is narrower than a global suite ambition, but it is more defensible, more margin-accretive, and more likely to support both public-market rerating and strategic optionality in future M&A scenarios.^{444 344 29}

References and Data Sources

13

References and Data Sources

This section consolidates the source base underpinning the report and distinguishes between primary evidence, contextual secondary materials, and the methodological conventions used to normalize heterogeneous disclosures across public and private health IT vendors. The source hierarchy prioritized audited company filings, official investor communications, government and regulator publications, and KLAS market intelligence, with secondary trade, analyst, and academic materials used only where they added necessary context or filled disclosure gaps.

13.1

Primary Sources

- Company filings and investor materials: The core financial, segment, recurring revenue, leverage, R&D, and geographic exposure analysis for CompuGroup Medical was anchored in the company's FY2024 and FY2025 annual reporting and official 2026 investor communications, including the FY2025 annual report and related press materials.^{100 102 444}
- Peer company disclosures: Publicly listed peer benchmarking relied on official filings and annual reports, especially for NEXUS AG and Oracle, because these provide the cleanest basis for revenue, EBITDA, maintenance or recurring revenue, and capital structure comparisons.^{36 235}
- Official company product and strategy pages: Product capability, cloud migration, AI positioning, and go to market evidence for private vendors were drawn from official vendor materials where audited financial disclosure was unavailable or incomplete. This applied particularly to athenahealth, eClinicalWorks, MEDITECH, Oracle Health, Dedalus, and CGM product releases.^{263 135 282 255 343}
- KLAS market intelligence: KLAS was treated as a primary market evidence source for customer satisfaction, acute care market share directionality, non U.S. hospital decision activity, and Best in KLAS recognition because it is based on direct provider feedback and validated purchasing data.^{26 157}
- Government and regulatory publications: Regulatory framing for interoperability, AI governance, and data sharing relied on official U.S. and EU sources rather than commentary, especially for ONC HTI rules, CMS interoperability requirements, the European Health Data Space, and the EU AI Act implementation context.^{311 78 69 71}
- Market and transaction primary disclosures: Where M&A activity or strategic developments were discussed, the report prioritized company press releases and transaction announcements over tertiary summaries.^{364 371 363}
- Market data providers used with caution: Public market capitalization and enterprise value references for listed peers were used only as date stamped valuation snapshots and were cross checked against company filings where possible.^{431 421 404}

13.2

Secondary Sources

- Trade and sector publications: Healthcare IT trade outlets were used selectively to contextualize KLAS findings, summarize vendor momentum, and capture market share commentary where

the underlying KLAS full datasets were not publicly accessible. These sources were treated as interpretive overlays rather than substitutes for primary evidence.^{117 27 119 468}

- Business and financial press: Publications such as Forbes, Bloomberg, and MedCity News were used to supplement private company scale estimates, leadership developments, and strategic positioning where audited disclosures were unavailable. These references were used conservatively and clearly distinguished from filed financial statements.^{144 239 123 148}
- Market research and macro industry reports: Top down market sizing and segment growth estimates were sourced from recognized commercial research firms and used only for directional framing because taxonomy differences across providers are material.^{49 50 52 51}
- Multilateral and public policy context: OECD and WHO materials were used to frame structural demand drivers, efficiency pressures, and digital health policy direction, especially where these shaped procurement behavior and long term sector attractiveness.^{58 59}
- Academic and technical literature: arXiv sourced papers were not used for company financial benchmarking, but they were used to support the report's methodological and technical interpretation of interoperability, cloud native clinical data architectures, AI workflow integration, and healthcare IT value creation under ownership change.^{250 253 451 4 359}
- Source exclusion principle: Academic preprints identified in the initial arXiv first search were useful for technical context but insufficient for the core evidentiary needs of this investor report, which required current company, market share, customer satisfaction, and transaction data through mid 2026. As a result, arXiv materials were retained as secondary context rather than as the report's primary factual backbone.^{450 3}

13.3

Methodological Appendices

- Source hierarchy and weighting: The report used a four tier evidence hierarchy. Tier one included audited annual reports, SEC filings, official investor releases, and regulator publications. Tier two included KLAS reports and official vendor announcements. Tier three included reputable trade and financial press. Tier four included academic preprints and commercial market research used for context rather than for core issuer level facts.^{100 36 26}
- Time boundary: All sourcing was constrained to the most current information available through mid 2026, with explicit preference for FY2025 company reporting, 2026 KLAS publications, and 2026 regulatory or transaction updates. Older sources were retained only where they remained the latest reliable disclosure for private companies or long cycle market structures.^{235 157}
- Peer set construction: The benchmark universe combined global acute care leaders, ambulatory cloud leaders, and European regional incumbents. Inclusion was based on one or more of the following criteria: meaningful hospital or ambulatory market share, relevance in DACH or Western Europe, strategic overlap with CGM in ambulatory, hospital, pharmacy, or interoperability workflows, or significance in current M&A and customer satisfaction discussions.^{117 343 36}
- Financial normalization: Revenue, EBITDA, recurring revenue, and R&D figures were reported in issuer disclosed currencies and periods, then compared on a like for like basis where possible. No synthetic margin or valuation estimates were created for private vendors unless a reputable source explicitly provided them. Where segment definitions differed, the report favored directional comparability over forced precision.^{100 36}
- Treatment of private company opacity: For Epic, athenahealth, eClinicalWorks, MEDITECH, and Dedalus, the report separated disclosed facts from inferred positioning. Financial scale,

valuation, and margin comparisons were only included when supported by official company statements or high credibility external reporting. Otherwise, benchmarking relied on market share, KLAS standing, product breadth, and strategic role.^{135 25 239}

- Market share methodology: U.S. acute care share references were based on KLAS derived public summaries and were interpreted as directional indicators of installed base concentration rather than exact universal census counts. Non U.S. hospital positioning relied on KLAS global decision data and regional vendor disclosures, especially where bed counts and facility counts were used as proxies for procurement gravity.^{117 363}
- Customer satisfaction methodology: KLAS awards, category scores, and migration commentary were used as the principal independent satisfaction and loyalty proxies because directly comparable public NRR, GRR, or churn disclosures are largely unavailable across the peer set. Where no suite level KLAS evidence existed for a vendor, the report explicitly noted the visibility gap rather than imputing a satisfaction ranking.^{26 343}
- Valuation methodology: Public market multiples were calculated from contemporaneous market capitalization or enterprise value snapshots and the latest available annual revenue or EBITDA disclosures. Oracle was treated only as a partial reference because its multiple reflects a diversified software and cloud platform rather than Oracle Health as a standalone asset.^{431 421 404 235}
- Replicability note: A future analyst reproducing this report should begin with the same source order used here, namely official filings, KLAS publications, regulator materials, official vendor releases, and only then secondary press and academic context. The main irreducible limitation is uneven disclosure by private vendors, which means some peer comparisons are necessarily asymmetric even when the analytical framework is consistent.^{100 26 250}



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